



Situation Analysis

LEBANON CRISIS

Period: 11/05/26 – 17/05/26

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Introduction

Purpose

This report provides a comprehensive overview of the current humanitarian situation, focusing on both operational environment factors and sectoral analysis. It aims to support informed decision-making for humanitarian partners and stakeholders.

About GANNET

GANNET is a Generative AI Initiative from Data Friendly Space, offering AI-powered tools built to make knowledge and real-time information accessible to our community in a resource-efficient and ethical way. It provides tools such as the GANNET Virtual Assistant and GANNET SituationHub to help humanitarian actors access, analyze, and utilize critical information for their operations.

GANNET SituationHub

The GANNET SituationHub is a generative AI-powered digital public good designed to identify and analyze humanitarian data from thousands of trusted sources in real time. By transforming complex data into clear insights, it enables faster, more effective response strategies and improved coordination among stakeholders.

GANNET Virtual Assistant

The GANNET Virtual Assistant is a verified chatbot designed for humanitarian quick decision-making. Built on cutting-edge Generative AI and trained with humanitarian expert-reviewed data, it provides real-time information. The GANNET Virtual Assistant facilitates rapid data access and synthesis for crisis response.

Methodology Overview

The Situation Hub's analytical process uses a structured Human-in-the-Loop workflow that combines automated analysis retrieval through GANNET Virtual Assistant, DFS's AI-powered humanitarian assistant, with expert review by the DFS analysis team. GANNET automates the extraction and generation of crisis-related insights based on structured prompts, which are aligned with a tailored analytical framework. These outputs are then reviewed, validated, and refined by analysts to ensure accuracy, contextual relevance, and operational utility. The workflow is organized into four key phases: planning and setup, ongoing operations, quality assurance, and system evolution, ensuring a balance between speed and analytical rigor.

Planning, Setup & Quality Control

In the planning phase, analysts develop a contextualized analytical framework, largely based on the Joint Intersectoral Analysis Framework (JIAF), and convert it into structured prompts for AI-assisted data extraction. These prompts are refined over time to improve output quality. Data sources are vetted for reliability, and translations are reviewed for accuracy. Using a centralized Content Management System (CMS), analysts review and curate AI-generated outputs, validate them against external sources, and make refinements to enhance clarity and relevance. The CMS enables collaborative editing and version tracking, ensuring outputs align with humanitarian needs.

Bias Mitigation

To minimize bias, the team diversifies data sources, cross-verifies information, and continuously evaluates the AI's performance. Analysts are crucial in identifying and correcting inaccuracies, especially in automated categorization. Future improvements aim to enhance source credibility checks and sentiment analysis, further strengthening the reliability of the Situation Hub's outputs.

Crisis Overview

The SituationHub landing pages use a standardized yet customizable template to deliver consistent, comprehensive crisis overviews. These pages integrate static content with dynamic data from sources like ACLED and DTM via open APIs, enabling interactive visuals with real-time updates. Regular reviews and verification with in-country partners ensure accuracy, while manual updates are used for sources not connected to APIs.

Media Monitoring

The media monitoring process has evolved from a manual, list-based approach to an AI-assisted system integrated into the CMS. This allows analysts to efficiently review, filter, and contextualize media narratives, while automation helps track emerging trends.

Overview

The following overview has been generated using the information available up to May 26, 2026. It provides a synthesized summary and key insights into the crisis based on the most recent data accessible at that time.

Summary

The conflict between Hezbollah and Israel, which began in October 2023 and escalated sharply in September 2024, entered a new phase in early 2026. Although a ceasefire came into effect on 27 November 2024, violations continued almost daily, with Israeli airstrikes and continued occupation of five positions in southern Lebanon preventing reconstruction and the return of displaced communities.

The ceasefire effectively collapsed on 2 March 2026 following U.S.-Israeli strikes on Iran. Hezbollah responded with rockets and drones targeting Israel, prompting heavy Israeli bombardment, expanded ground operations, and evacuation orders across southern Lebanon and Beirut's southern suburbs.

UNIFIL has faced severe operational constraints during its final mandate year. Since March, peacekeepers have been killed, patrols targeted, and monitoring infrastructure damaged, undermining the mission's ability to document violations along the Blue Line. Israeli military operations intensified throughout March and April 2026, evolving from "limited" operations into a broader ground campaign extending deeper into southern Lebanon.

Since the massive military escalation that spiked on April 8, 2026, Israel has targeted thousands of sites across Lebanon, including hitting over 100 concentrated targets on April 8 alone. A breakthrough in mid-May resulted in a 45-day ceasefire extension and a roadmap for further military and political negotiations, although Hezbollah continued to reject the process and sporadic violence persisted. As a result, the diplomatic deal remains highly fragile, with continued localized airstrikes, rocket fire, and civilian casualties on the ground.

The humanitarian impact has been catastrophic. Since 2 March 2026, over 3,070 people have reportedly been killed and nearly 9,362 injured, with women, children, and the elderly accounting for an estimated 25% to 35% of the total civilian casualties. The heaviest structural destruction is concentrated in Southern Lebanon, the Bekaa Valley, and the southern suburbs of Beirut, where residential blocks, key roads, schools and vital healthcare infrastructure have been flattened, forcing over 1.2 million people to flee their homes.

Humanitarian funding in Lebanon remains critically inadequate. By late May 2026, the UN's USD 308.3 million Flash Appeal for March – May 2026 had secured only 51.3% of the required funding, leaving significant gaps in emergency relief and protection services. The funding shortfall threatens essential services, including food assistance, clean water, and healthcare support for the more than 1.2 million people displaced by the ongoing conflict.

Key Insights

1. The resumption of large-scale hostilities on 2 March 2026 marked the most severe deterioration in Lebanon's security since the November 2024 ceasefire. The escalation followed months of repeated ceasefire violations, with UNIFIL documenting over 10,000 air and ground breaches and at least 331 deaths before fighting fully resumed. Since March 2026, over 3,070 people have been killed and 9,362 injured, with women, children, and the elderly accounting for an estimated 25% to 35% of the total civilian casualties, while more than 1.2 million people have been displaced. While Israel has not launched a single, massive 24-hour campaign as concentrated as the April 8 wave, it has maintained a relentless, daily bombardment strategy throughout April and May 2026.

2. A diplomatic process began after the 8 April 2026 strikes, marking the first high-level Israel-Lebanon engagement since 1993. Preparatory talks on 14 April 2026 at the U.S. State Department, led by Secretary of State Marco Rubio, revealed opposing positions. Israel demanded Hezbollah's disarmament, stronger enforcement of UN Resolution 1701, and security guarantees for northern Israel, while Lebanon called for an end to Israeli airstrikes, withdrawal from southern Lebanon, release of detainees, return of displaced civilians, and reconstruction aid. A second round on 23 April 2026, chaired by President Donald Trump, led to a breakthrough in talks on 14–15 May 2026, with agreement on a 45-day ceasefire extension and a roadmap for further negotiations. However, Hezbollah rejected the process as a "concession" to Israel and continued cross-border attacks during negotiations.

3. The conflict is heavily rooted in the wider 2026 U.S. Iran war, but Lebanon's diplomatic fate has recently decoupled from it. While the disputed April 7 U.S. Iran truce initially left Lebanon highly vulnerable to regional escalation, the resulting crisis pushed Washington to mediate direct, high-level talks between Israel and the Lebanese government. This parallel track achieved a major breakthrough on May 15, 2026, securing a 45-day ceasefire extension and a structured military political roadmap. While the broader U.S. Iran negotiations remain deadlocked, Lebanon has carved out its own distinct diplomatic track, though the ground reality remains highly volatile due to isolated cross-border skirmishes and Hezbollah's rejection of the bilateral process.

4. Between 2 March and mid-May 2026, the Lebanon conflict displaced over 1.2 million people, forcing 20% of the population (1 million internal refugees) from their homes and sending 300,000 fleeing into Syria. Although a mid-April truce was extended for 45 days in mid-May, returnees were immediately forced back into displacement after finding their border villages structurally leveled and cut off from water and electricity. Humanitarian agencies are stretched to their limits; the UN's \$308.3 million Flash Appeal is only 51.3% funded by late May, severely crippling the response framework as needs continue to surge.

5. Food insecurity was already projected to worsen before the renewed escalation. A joint FAO/WFP/Government IPC analysis found that approximately 874,000 people (17% of the population analyzed) faced acute food insecurity between November 2025 and March 2026, with projections indicating a further rise to approximately 961,000 people, or 18% of the population, between April and July 2026. The renewed conflict, which has cut humanitarian access and re-displaced more than 1.2 million people, is expected to sharply exceed

those projections.

6. The March 2026 escalation has significantly reversed displacement gains and risks conflating two fundamentally different population movements. Before 2 March, Lebanon was experiencing the largest Syrian voluntary return movement since the conflict began: UNHCR inactivated 552,413 Syrian cases due to confirmed or presumed return since January 2025, underpinned by Lebanon waiving exit fees and re-entry bans from July 2025. Since the resumption of hostilities, more than 450,000 people in total, including nearly 200,000 Syrians and over 28,000 Lebanese, have fled across the border under bombardment. UNHCR's ability to ensure returns are safe, informed, and truly voluntary is therefore severely compromised due to the active conflict environment. With Lebanon hosting approximately 1.3 million Syrian refugees, the highest per capita globally, and conditions in Syria remaining fragile, the March escalation threatens to lock a deeply vulnerable population into a cycle of compounded, potentially irreversible displacement with no viable protection framework on either side of the border.

7. Lebanon's economy had registered its first growth since 2017 — the World Bank recorded real GDP expansion of 3.5% in 2025 — but the March 2026 escalation has placed that fragile recovery in severe jeopardy. The IMF's April 2026 World Economic Outlook notes that conflicts generate large and persistent output losses exceeding those from financial crises or natural disasters, and Lebanon enters this escalation with a banking sector carrying over USD 72 billion in accumulated losses and public debt at 176.5% of GDP. The conflict has directly struck the pillars of the 2025 recovery: the south of the Litani has been fully severed from the rest of the country since 16 April, collapsing market activity and cutting supply chains entirely; the spring tourism season is effectively lost; and the escalation delays IMF reform requirements, deters investment, and risks prolonging stagnation well into 2026 and beyond. Lebanon's path to an IMF program, of which only the bank secrecy amendment has been adopted from 8 required prior actions, is now contingent on a peace that does not yet exist.

Economy

Current Economic Situation

Lebanon is experiencing a deepening macroeconomic shock: Economy Minister Amer Bisat estimated the war has caused an economic loss of around 7% of GDP and described the shock as existential [4]. The economy contracted sharply by 7.1% in 2024 before a partial recovery estimated at +3.5% in 2025, while cumulative financial-sector losses since 2019 have been put at about \$70 billion with additional war-related losses cited around \$11 billion [29][4]. The Lebanese pound has lost over 90% of its value against the US dollar since 2019, and the banking collapse evaporated depositors' funds; the sector remains unresolved, with depositors still waiting for access to blocked funds and authorities yet to finalize sector restructuring [4][1]. Inflation accelerated to 17.26% year-on-year in March 2026 (from 12.27% in February) with a month-on-month increase of 4.91%, and purchasing power remains weak as households rely on credit and cut non-food spending [1][55].

Key industries are severely affected: tourism occupancy rates were reported at roughly 5–10% overall and about 7–8% in Beirut for the Adha/summer period, many businesses report collapsed revenues (some restaurants cited ~90% drops), and widespread reliance on private diesel generators increases vulnerability to fuel price spikes [50][26][4]. Direct and indirect war losses have been estimated at over \$25–26 billion, adding major reconstruction pressures on public finances and the private sector [22][28]. Food insecurity is worsening, with an estimated 1.24 million people projected to face acute food insecurity between April–August 2026 and FEWS NET warning that staple food and fuel prices will remain significantly above average through September 2026 [1][89][55]. Humanitarian financing gaps are constraining the response: the 2026 Lebanon Flash Appeal was only around 38–42% funded in early–mid May (roughly US\$117–129 million of the US\$308 million requested), limiting partners' capacity to sustain services [18][51].

Post-Conflict Economic Landscape

Lebanon faces large-scale reconstruction needs and acute economic disruption. The World Bank estimated \$11 billion in recovery and reconstruction needs (physical damage \$6.8 billion; economic losses \$7.2 billion) for October 2023–December 2024, while Lebanese private-sector estimates place direct and indirect war losses at about \$25–26 billion, with roughly \$12 billion identified as immediate reconstruction needs and indirect losses of about \$30 million per day cited by business leaders [44][15][2][22]. The World Bank forecasted up to 4% real growth for 2026 conditional on reforms and reconstruction financing [37][1].

Key sectoral impacts include:

- Housing and infrastructure: housing was the most affected sector with an estimated \$4.6 billion in damage; more than 10,000 housing units have been reported as destroyed or affected [15].
- Agriculture: direct agricultural damage is estimated at ~\$118 million, losses at ~\$586 million (total >\$700 million), and reconstruction needs at ~\$263 million (with \$95 million priority for 2025–2026); about 54,000 hectares were directly affected (~22.5% of national agricultural area), with large unharvested or late-harvest areas and damaged irrigation and cooperatives reported [44].
- Displacement and food security: over 1.2 million people were forced from their homes between March and April; acute food insecurity is estimated at ~1.24 million people for April–August 2026 (up from ~874,000 in November 2025–March 2026), with southern districts facing the highest severity [2][1].
- Humanitarian financing: the UN flash appeal (US\$308.3 million) was only ~38% funded, leaving major gaps as needs deepen [11].

The shock is concentrated in southern governorates — particularly Nabatieh and South — where agriculture, housing and basic services are most affected, posing medium- to long-term risks to livelihoods and return. Reconstruction faces four principal obstacles: access and clearance, documentation of damage, financing capacity, and prioritization of repair versus rebuild [15][11][44]. Diaspora remittances and IOM-supported diaspora engagement remain important sources of emergency support and technical assistance for recovery [84].

Stability of Currency and Bank System

The Lebanese pound remained officially at LBP 89,500 per USD in Bank of Lebanon data available until February 2026 [1]. Annual inflation rose to 17.26% in March 2026 (from 12.27% in February 2026) with a month-on-month increase of 4.91% [1]. Parallel market pressures and large official-to-parallel exchange-rate divergences are creating economic uncertainty and reducing the local-currency value of USD transfers for UN agencies, which are typically required to convert at the official rate; WFP's exchange-rate monitoring reports parallel rates for Lebanon using Lira Rate [35]. Market reports noted shortages of both Lebanese pounds and US dollars in domestic markets as of 11 May 2026 [47].

The banking sector remains unresolved since the 2019 collapse that evaporated depositors' savings, leaving many depositors waiting for access to blocked funds while sector restructuring and saver protections remain pending [4][1]. The renewed conflict has compounded fiscal stress, with private estimates placing cumulative war losses at roughly \$25–26 billion [22][28].

Employment

The ongoing hostilities and economic shocks have caused widespread job losses across tourism, agriculture, retail and informal day labour, with traders closing shops, farmers abandoning land and many daily workers losing income [4][1][23]. The hotel sector reported about 40,000 employees with unemployment among them estimated at 30–40%, current occupancy at 5–10%, and around 70% of hotels from Beirut airport to the south closed [50].

In southern Lebanon, more than three quarters of farmers have been displaced and significant agricultural area has been directly affected, undermining rural employment and seasonal work opportunities [1][44]. Key informant data show heavy reliance on informal work: primary income sources were reported as daily labour (75%), salaried employment (39%) and agriculture (38%), while over half of respondents said earnings did not meet basic needs [23].

The public education workforce has also been heavily affected: teachers' real incomes fell by about 80% since 2019 and roughly 30% of the sector left the country or changed professions, reducing formal public-sector employment in education [81]. Displaced people — over 1.04 million self-registered IDPs — face acute livelihood loss, with many hosted outside formal shelters [6]. Humanitarian cash support has been scaled to partially offset income loss: the Government-led Shock Responsive Social Safety Net disbursed about USD 15.6 million to 138,000 households, and UNHCR/WFP and partners are scaling emergency multi-purpose cash assistance (MPCA), while UNHCR has issued 2,000+ emergency protection cash grants reaching roughly 9,700 people including over 700 persons with disabilities [18][34][82]. Evidence from cash-plus programming indicates that while cash cushions immediate needs, such interventions have limited sustained impact on employment where labour demand is weak and economic volatility persists [46].

Politics

Governance Structures and Rule of Law

Lebanon's governance landscape remains severely strained by the interplay of active hostilities, institutional fragmentation, and the entrenched role of non-state armed actors. Prime Minister Nawaf Salam has publicly documented war crimes and stated that 68 villages remain under Israeli control, while Speaker Nabih Berri has set political conditions for any ceasefire — demanding Israeli withdrawal, reconstruction funding, deployment of the Lebanese army, and the return of inhabitants as prerequisites for restoring state authority [138][134][204]. Authorities acknowledge limited state control on the ground, noting that Hezbollah remains a major armed actor that refuses direct negotiation with Israel and constrains the state's ability to implement security arrangements [138]. The United States announced a 45-day extension of the Lebanon–Israel ceasefire and the launch of a U.S.-facilitated security track, which the Lebanese negotiating delegation said provides a margin for stability provided commitments are implemented through a phased, verifiable mechanism backed by American guarantees [190].

The Minister of Information, Paul Morcos, reported 1,744 ceasefire violations and presented a government survey covering 17 April–8 May to document incidents for international and legal purposes, linking this effort to diplomatic briefing needs [143]. The Ministry is coordinating awareness and training materials with UNESCO and UNDP to address hate speech and media practices, and a modern information bill that would regulate electronic sites and criminalize defined forms of hate speech is awaiting parliamentary adoption [143]. OCHA reported significant displacement dynamics during the ceasefire period: an initial 21% drop in registered shelter population to 117,421 across 631 sites was followed by rebounds, with around 90% of displaced people outside formal shelters; fresh displacement orders were issued for 11 localities on 4 May and overnight orders affected Sohmar on 11 May, while Israel warned people not to return to 74 localities [184].

Lebanon's social protection and service-delivery landscape remains fragmented and clientelist. The National Poverty Targeting Programme expanded after the 2019 economic collapse, but access continues to be shaped by political affiliation and local networks, and cash assistance is described as central yet limited in producing sustained economic outcomes [140]. Programmatic reviews from Tripoli underscore that weak labour markets, informality, and political-economic exclusion constrain the effectiveness of cash-plus interventions, pointing to the need for policy engagement and structural advocacy to address governance-related barriers to service and livelihood recovery [153].

Political Freedoms and Recent Laws

Lebanese authorities and statements from Beirut link the resolution of pending cases — including the release of detainees and the return of internally displaced persons — to implementation of the ceasefire and related security arrangements [114]. The National Council for Scientific Research in Lebanon (CNRS-L) reported that more than 10,000 homes have been destroyed or damaged since the truce, a figure Lebanon says undermines civilian protection and complicates the return of displaced persons and reconstruction planning [111][114]. Hezbollah's armed presence continues to divide the Lebanese political scene, challenge the state's monopoly on force, and complicate U.S. and Lebanese efforts to secure disarmament or a unified security framework [187][174].

Humanitarian and programme analyses highlight that Lebanon's social protection and access to assistance remain fragmented and are often mediated by political affiliation and clientelist networks, limiting equitable state-led provision and potentially affecting equal access to reconstruction and electoral participation opportunities [140][153]. Domestically, the Information Ministry is pursuing a draft law against hate speech alongside parallel documentation efforts to address internal polarisation — both measures aimed at strengthening civic space and restoring confidence in state institutions [143].

Influence of Non-State Actors and Political Factions

Hezbollah functions as a dominant non-state armed and political actor that constrains state authority, shapes Lebanon's political paralysis, and risks gaining strength domestically as Lebanese leaders both criticise and lack the capacity to disarm it amid ongoing Israeli operations [174][122]. Iran exerts significant influence through its ties to Hezbollah and regional manoeuvres to preserve its foothold in Beirut, while external actors — notably the United States and Gulf states — pressure Lebanon to curb Hezbollah, complicating domestic decision-making and negotiations [194]. Lebanon's political landscape is highly fragmented, with sectarian divisions and elite clientelism undermining unified state responses and increasing reliance on alternative power centres and local networks [174][140]. The interaction of a powerful armed political actor, external patronage, and entrenched clientelist networks is a core driver of governance dysfunction, constraining humanitarian access, service restoration, and coherent recovery planning [174][194][140][115].

Key operational impacts of this environment include:

- Since 2 March, 161 recorded incidents affecting healthcare have resulted in 110 deaths and 252 injuries among healthcare workers, with multiple facilities closed as of 14 May [115][108].
- Displacement remained acute: over one million IDPs in Lebanon as of 11 May, including approximately 121,701 in 628 collective sites; by 14 May, roughly 129,721 people were sheltering in 632 collective shelters [107][115].
- More than 10,632 housing units were reported damaged or destroyed since the truce, politicising returns and reconstruction planning [111][114].

Municipalities and local authorities are central to needs identification and return facilitation but lack fuel, equipment, and liquidity, increasing the need for decentralised coordination to enable safe, dignified returns [111][115].

Public Trust in State Institutions

Public trust in Lebanese state institutions has been significantly eroded by high-frequency hostilities, political fragmentation, and the state's limited capacity to enforce security arrangements. Political fragmentation constrains state legitimacy, as Hezbollah's refusal to negotiate directly with Israel and its retention of arms limit the state's monopoly on force, while President Aoun, Speaker Nabih Berri, and Prime Minister Nawaf Salam press for a unified state role and clear ceasefire conditions [122][138]. Municipal authorities in southern areas such as Tyre face acute operational pressure from strikes and displacement notices, further undermining confidence in local public security [143][181].

International engagement is being leveraged to bolster government credibility, with the UN humanitarian coordinator calling Lebanon-Israel talks a "critical opportunity" to end the war and facilitate withdrawal guarantees [203]. In parallel, the Information Ministry's pursuit of a draft law against hate speech and its documentation efforts represent domestic initiatives aimed at addressing internal polarisation and rebuilding public trust in institutions [143].

Role of Marginalized Groups

In Tripoli, cash-plus programmes combining temporary cash transfers with vocational training found that labour-market saturation, informality, and limited job creation constrain the translation of short-term gains into sustained income for marginalized groups [153][140]. These interventions nonetheless generated psychosocial and relational gains – including increased confidence, social participation, and shifts in intra-household dynamics among women – which programmes identified as important dimensions of economic participation [153]. Programme design features such as accessible locations and group-based formats supported participation within these constrained conditions [153]. Cash transfers for Syrian refugees were systematised at scale from 2017 through UNHCR and WFP, providing regular unconditional support that improved immediate food security and basic expenditures but showed limited evidence of long-term economic outcomes without sustained support [153]. Analyses recommend linking programme delivery to policy engagement with municipalities and local institutions to address structural barriers to employment and service access and to sustain participation gains beyond short-term stabilisation [153].

Security developments in southern Lebanon on 12 May 2026 included Israeli forces crossing the Litani River and establishing "operational control" over Zawtar al Charqiyeh in Nabatieh District while conducting clearing operations targeting Hezbollah infrastructure, a dynamic that affects civilian safety and local governance in border areas [157]. Local alerts in South Lebanon highlight risks to municipal services and displacement sensitivities near Tyre, where the presence of Palestinian camps amplifies protection and displacement concerns amid military and settler-movement pressures [181][169].

Security And Stability

Security Overview

The security situation in Lebanon remains highly volatile, with ongoing Israeli strikes, armed activity by Hezbollah, evacuation alerts in southern localities, and widespread presence of weapons outside state authority that undermine civilian safety and humanitarian operations [263][237][271]. UN officials have warned the ceasefire is increasingly fragile, with UNIFIL reporting six peacekeepers killed since 2 March, and Lebanese authorities and AFP tallies putting the country's overall death toll since strikes began at over 2,800, including more than 400 fatalities during the ceasefire window [263][271]. Attacks on health services continued, with six new attacks recorded in the latest reporting period resulting in five health-worker deaths and eight injuries, including reported strikes on 11 May that targeted paramedics in Toul and hit a health facility in Srifa (one killed, four injured) [259]. Humanitarian operations continue to face impediments from damaged infrastructure, disrupted supply pipelines, and unexploded ordnance contamination that restrict safe returns and service delivery [259].

Displacement levels remain elevated with no large-scale or sustained returns observed in South Lebanon and Nabatieh; collective shelter populations are approaching pre-ceasefire peak levels, with protection actors reaching 182,313 IDPs as of 11 May and 192,600 IDPs as of 14 May [259][272][256]. Children remain disproportionately affected, with an estimated 390,000 displaced children and heightened risks of family separation, exploitation, and interrupted services [256]. Key operational response figures include:

- Protection services: 64,976 people received MHPSS, 1,259 individuals received case management, 1,796 protection cash grants were provided, 24,841 women/girls received dignity kits, and ~21,000 people were supported through protection feedback/complaints channels [272].
- Shelter/WASH response: repairs/upgrades underway across 360 collective shelters (282 completed to date), WASH assistance delivered in 611 collective shelters since 2 March, 88,043 hygiene kits and 50,333 menstrual kits distributed, and 3,765,725 litres of potable water delivered [272].

Security Forces and Law Enforcement

Since mid-May 2026, the Lebanese Armed Forces (LAF) and internal security forces have been deployed on roadblocks and checkpoints to promote stability and protect displaced communities amid heightened security measures [213]. Lebanese negotiators seek formal LAF deployment south of the Litani as part of ceasefire implementation and territory return, with draft texts envisaging the LAF assuming full security responsibility over returned areas if Israel withdraws; UNIFIL's mandate is due to expire on 31 December 2026, with post-UNIFIL options to be explored by 1 June 2026 [231]. The United States and international partners have proposed training and equipping programmes to strengthen the LAF's capacity to consolidate state authority and enable safe returns, as reflected in negotiation drafts [231][232].

State control on the ground remains limited because Hezbollah continues to operate as a major armed actor and has not joined the Washington talks, complicating efforts to centralize the monopoly of force under state institutions [242][214][219]. Operational capability and humanitarian access are constrained by an Israeli-declared military zone in the south, damaged infrastructure, IED/UXO contamination, and disrupted communications, which limit movement, increase risks to personnel, and complicate security operations [272]. Civil-military coordination has allowed some humanitarian access, with 121 Humanitarian Notification System-facilitated movements enabling delivery of life-saving assistance across southern Lebanon between 2 March and 11 May, though access is increasingly contingent on route predictability and local security conditions [272]. Verified attacks on health facilities and personnel have further degraded security-sector capacity to protect civilians and responders, with WHO-verified figures recording 158 attacks on healthcare causing 108 deaths and 249 injuries as of 11 May 2026, and field reports confirming severely constrained service delivery in affected areas [221][233].

Demography

Demographic Overview

Lebanon's population is roughly 5.5 million people [294]. Since the escalation on 2 March, over 1.04 million people have self-registered as displaced on the Government of Lebanon relief platform (MoSA), with overall displaced estimates exceeding one million by May 2026 [297][295]. Most displaced households are hosted outside formal shelters—in host families, rented accommodation, or informal arrangements—placing sustained pressure on host communities and complicating humanitarian planning [317][292]. The collective-shelter population has trended upward, rising from approximately 125,621 people on 6 May to roughly 127,210 by 11 May, and reaching approximately 129,721 people across 632 collective sites by 14 May 2026—representing a 13% increase in IDPs in shelters since 17 April 2026 [297][295][318][292]. Children represent a large share of the displaced, accounting for about 35% of IDPs, with an estimated 390,000 displaced children nationally and roughly 44,800 children currently in collective shelters [290][320][292].

Cross-border movements remain substantial: IOM/DTM recorded some 312,996 movements at official points of entry as of 11 May 2026 (approximately 96% Syrian nationals), with UN/IOM sources reporting roughly 321,666 Syrians crossing back to Syria between 2 March and 6 May, while around 38,000 people have returned to Lebanon in the same period [306][297][307].

Health sector reporting (MoPH/WHO) through 15 May indicates 11,939 total casualties since 2 March, including 2,951 deaths and 8,988 injuries. Casualties are disproportionately male (approximately 86%), with children accounting for about 9% of casualties (209 child deaths and 823 injured). The vast majority of casualties are reported as Lebanese (approximately 94%), with burdens concentrated in Nabatieh and South governorates, followed by Mount Lebanon [312][292].

Socio-Cultural

Socio-Cultural Overview

In many parts of Tripoli, host and refugee communities are socially, economically and geographically intertwined with shared norms and religious affiliation, and while tensions surface under scarcity, long-standing ties complicate a binary host/refugee framing [338]. Access to social protection and assistance is fragmented and often mediated by political affiliation and local networks rather than solely by need, despite the National Poverty Targeting Programme's expansion since 2019 to include cash support [332]. Displacement is reshaping community composition: families from predominantly Shia southern areas have settled in localities with different religious fabrics, increasing concerns about inter-community tensions and pressure on urban services and housing stock [340][339]. Community relations are described as fragile but include local examples of mutual assistance and host support for livelihoods, even as competition over rents, water and generators fuels tensions in host cities [333][340]. Elevated social tensions are further driven by host-community pressure, shelter capacity shortfalls, intra-Lebanese polarisation and misinformation, and return movements following ceasefire announcements were often short-lived in May 2026 [347]. Local solidarity persists—municipalities, volunteers and diaspora-linked initiatives support shelter, food and recovery—but municipal capacities are overstretched and lack fuel, equipment and liquidity to manage diverse damage and shelter needs [336][371]. Local informal support networks, community committees and volunteer initiatives remain central coping mechanisms and channels for outreach and information sharing, while spontaneous community solidarity (e.g., food preparation, ad hoc clinics) has been reported in affected areas [347][382]. Education has been severely disrupted—schools converted into shelters and large-scale displacement are contributing to learning loss and widening inequalities among students [341][337]. Widespread anxiety, trauma and rising psychosocial needs have been reported among displaced children and families, with UNHCR estimating that children constitute about 35% of IDPs in Lebanon [331][350][384].

Gender Dynamics

Protection actors report heightened protection risks for women and girls. As of 11 May, 77 Protection, Child Protection and GBV partners reached 182,313 IDPs; GBV partners provided 24,841 women and girls with dignity kits, 1,259 people with dedicated case management, 1,796 protection cash grants, and 64,976 people with MHPSS interventions [347]. By 14 May, partners had reached 192,600 IDPs, and GBVIMS analysis indicated that ongoing and repeated displacement is disrupting continuity of care and complicating case follow-up

[335]. Community-based protection reached 9,724 displaced people since 2 March, 63% of whom were women and girls [333]. Identified GBV risks include increased domestic violence linked to economic stress, harassment restricting women's mobility, and rising early marriage as a coping mechanism [333]. A nationwide GBV assessment with data collection closed on 8 May was launched to inform prioritization, with publication expected by end-May [333]. Women and girls — particularly those living in tents, public spaces or outside collective sites — and persons with disabilities remain among the most at-risk due to unsafe accommodation, lack of WASH and disrupted services [333][335]. Nutrition and health assessments (March–May 2026) identified approximately 12,200 pregnant women displaced nationwide, including at least 675 in collective shelters, underscoring acute risks for pregnant and lactating women [373]. Funding shortfalls, access constraints and fragmented referral pathways are limiting the scale-up and continuity of GBV and protection services across affected areas [347][335][339].

Impact of Socio-Cultural Drivers on Aid

Cultural norms, social hierarchies and caregiving responsibilities shape both vulnerability and access to humanitarian assistance. Oxfam's evaluation of cash-plus programming in Tripoli shows that gender norms, caregiving responsibilities and limited female mobility shape participation in training and employment transitions, with women prioritising proximity and childcare over available job offers [338]. IWMI's Al Murunah pilot highlights the influence of local opinion leaders and the need for conflict-sensitive, locally-led engagement to secure inclusive participation and adapt implementation under security and mobility constraints in Lebanon [370]. Protection risks are noted as disproportionately acute for women and girls, separated children and persons with disabilities, which constrains safe access to assistance and services across displacement sites [347]. Key nutrition and caregiving gaps identified by Action Against Hunger (March–May 2026) include: 38% of collective shelters lack nutrition support coverage, with the largest gaps in Beirut and Mount Lebanon [373]; infant and young child feeding is severely disrupted, with exclusive breastfeeding under 6 months at only 7% in Saida, 16% in Baalbek–Hermel, and 24% in Beirut and Mount Lebanon, while reliance on breastmilk substitutes reaches 85% in Saida, 59% in Beirut and Mount Lebanon, and 38% in Bekaa and Baalbek–Hermel [373]. Mass protection messages reached approximately 90,000 people and 21,000 protection feedback and complaints were processed as of 11 May [347].

Environment

Environmental Conditions

Conflict-related environmental damage is concentrated in southern Lebanon, with substantial agricultural and forest loss documented through 2024–2025 and renewed strikes causing further harm in 2026 [426][411]. Explosions, fires and large-scale demolition have produced vast volumes of rubble and airborne dust and soot, dispersing hazardous materials including asbestos and heavy metals, with glyphosate also detected after a 2026 episode near the border [426][434][411]. A major marine pollution event from the Jieh power-plant bombing spilled approximately 15,000 tonnes of oil and contaminated roughly 150 km of coastline [426]. Damage to irrigation networks, orchards and wells — together with watershed degradation, deforestation, erosion and increased flash-flood risk — has intensified water scarcity and undermined agricultural livelihoods and prospects for safe return [411][428][431]. Access restrictions, ongoing hostilities and unexploded ordnance are impeding comprehensive soil and water testing and rubble clearance, delaying identification of land safe for cultivation, drilling or reconstruction [457][459][411][456]. Independent environmental monitoring (soil and water analyses), restoration of irrigation systems, and clearance of unexploded ordnance are highlighted as priority indicators to determine where it is safe to rebuild, farm or drill wells [411].

Environmental Challenges

Conflict-related attacks since 2023 have caused concentrated environmental damage in southern Lebanon, with ministerial and scientific reporting estimating about 54,000 hectares of agricultural land directly affected [411][426]. Key damage figures include:

- 5,000 hectares of forest cover and roughly 2,154 hectares of orchards destroyed, with 54,000 hectares of agricultural land affected overall [426][411].
- Hundreds of white-phosphorus fires (812 reported October 2023–June 2024) and measured soil contamination, with phosphorus recorded at approximately 1,858 ppm in impact assessments [426].
- Debris volumes estimated in the tens of millions of tonnes (reported ranges of ~50–100 million tonnes), with widespread UXO and cluster munition contamination noted in affected areas [426][434][425].

Rubble and war waste contain hazardous materials — asbestos, paints, solvents, heavy metals and fuel remnants — alongside unexploded ordnance, posing long-term soil, water and air pollution risks and complicating safe clearance and disposal [411][434]. Debris-management and reconstruction choices carry further environmental risk: international assessments note Lebanon's post-conflict experience using rubble for coastal reclamation but warn of long-term environmental and governance controversies, and UNEP and UN actors stress strict sorting, hazardous-waste removal and environmental and social impact assessment (ESIA) controls before any coastal or landfill use of rubble [434][424].

Deterioration of WASH and shelter environments — including water leakage, sewage contamination, damp, soot and inadequate sanitation in collective sites — is driving increased exposure to waterborne and respiratory illnesses and raising malnutrition risks among displaced populations, as documented in nutrition and protection assessments in March–May 2026, while WHO continues to monitor environmental and CBRN risks from strikes on industrial and water infrastructure [447][444][442].

Environmental Risks and Mitigation

Since early May 2026, renewed strikes and displacement have continued to generate contaminated debris, unexploded ordnance (UXO) and damage to water, sanitation and energy infrastructure, threatening WASH continuity, complicating rubble removal and delaying safe returns and recovery operations [419][436].

Unexploded ordnance and contamination are repeatedly cited as primary barriers to cultivation and return, requiring prolonged submunition and EOD clearance and specialized environmental remediation before safe reconstruction can proceed [411][426].

Humanitarian mitigation and technical measures underway or recommended include environmental and social management plans (ESMPs) and systematic environmental risk screening in programmes – NRC reported ESMP and NEAT+ use in 2025 – targeted asbestos removal where identified, explosive ordnance clearance priorities, and WHO-led CBRN monitoring and preparedness actions, including a radiological hazards webinar for Lebanese emergency response physicians on 15 May 2026 to strengthen health-sector readiness [423][411][420][448]. Guidance on reuse of conflict debris and coastal reclamation stresses mandatory ESIA, strict material screening and quality control, worker health and safety, and ongoing environmental monitoring, drawing on lessons from Lebanon's prior rubble reclamation experience and UNEP recommendations for robust safeguards [424][434]. National technical reports and assessments estimate overall environmental damage in the multi-billion USD range [426][411][425].

Conflict

Overview of the Ongoing Armed Conflict

The Israel–Hezbollah escalation began on 2 March 2026, leading to a ceasefire announced on 17 April 2026, which was subsequently extended for 45 days following U.S.-mediated talks on 15 May 2026 [539][499][588]. Violence remains concentrated in southern Lebanon—particularly Nabatiyeh, Tyre, Bint Jbeil, Marjayoun and Jezzine—while hostilities have also affected parts of the Bekaa Valley and Beirut's southern suburbs [539][526][567]. Hostilities have continued despite the truce: WHO reported 14,674 total incidents from 2 March through 14 May, including 5,503 incidents after 17 April, and MoPH cumulative figures reached 11,939 casualties including 2,951 deaths as of 15 May 2026 [539]. UNICEF and Lebanese health authorities report heavy child casualties, with roughly 200 children killed and 806 injured since 2 March and at least 23 children killed and 93 injured since the 17 April ceasefire [576][586]. Attacks on health care have persisted: WHO's SSA data record 259 health-care worker injuries and 110 deaths since 2 March, WHO verified 15 attacks on health care resulting in 12 deaths and 21 injuries as of mid-May, and multiple hospitals have been damaged, with three remaining fully non-operational and sixteen partially damaged [519][555]. Displacement and shelter needs remain acute, with more than one million people displaced nationally, roughly 127,000–129,700 people sheltering in some 629–715 collective sites, and renewed displacement orders for about 27 localities driving additional movements and rising pressure on shelters since mid-April [555][519][583]. Humanitarian access is increasingly constrained by ongoing insecurity, route unpredictability, UXO/IED contamination, telecommunications disruptions and risks of pipeline breaks for health and WASH supplies [583]. UN officials have highlighted the continued presence of weapons outside Lebanese state authority, ongoing Hezbollah strikes and retaliatory Israeli operations—together with attacks that have killed UNIFIL peacekeepers—as factors sustaining instability and the risk of renewed escalation [561].

Conflict Stakeholders

Key state and non-state parties are the Government of Lebanon and the Lebanese Armed Forces (LAF), Hezbollah, and Israel, with the United States acting as lead mediator and the UN/UNIFIL and major EU actors engaged in security and reconstruction discussions [597][522][601][561]. Hezbollah remains the decisive armed non-state actor: it is absent from Washington negotiations but continues military operations on the ground, complicating disarmament ambitions and the implementation of any ceasefire or territorial handover [522][512]. Israel's objectives focus on neutralizing Hezbollah and maintaining an extended buffer/operational zone north of the border, while Lebanon's official negotiating priorities are cessation of strikes, Israeli withdrawal and the safe return of displaced persons [522][528]. The United States is pushing a sequencing that reinforces the LAF—including training and equipping programmes and handover of occupied areas—as outlined in draft negotiation texts circulated during mid-May talks [597][528]. Iran exerts regional influence through support for Hezbollah and IRGC-linked operatives, prompting Lebanese authorities to pursue measures including arrest warrants and public distancing from Iranian actors [594]. Six UNIFIL peacekeepers (four Indonesian, two French) were killed since 2 March, highlighting the direct risk to international forces on the ground [561]. Humanitarian and UN actors are operational on multiple fronts: UNHCR maintains country and field offices, and humanitarian partners using the Humanitarian Notification System facilitated 131 movements and delivered large-scale WASH and relief items—including 95,954 hygiene kits, 52,803 menstrual kits, 3,765,725 litres of potable water, 76,090 m³ of water trucking, and 803,000 litres of fuel sustaining public water for 656,973 people—between 2 March and 14 May, though access remains constrained by insecurity, UXO contamination, GPS/communications disruptions and damaged routes [602][542]. The security dynamic remained fluid during the 14–15 May Washington talks, with ongoing strikes, reported Israeli claims of substantial Hezbollah losses and expanded operational perimeters in southern Lebanon continuing to shape negotiating leverage and local civilian protection risks [609][528].

Peace Negotiations

Lebanon and Israel agreed to extend their 17 April ceasefire by 45 days after two days of U.S.-mediated talks in Washington, with a Pentagon security track set for 29 May and a State Department political track scheduled for 2–3 June [468][483][497]. The talks involved Lebanese government delegations (including special envoy Simon Karam and ambassador Nada Hamadeh), Israeli officials (including Ambassador Yechiel Leiter and deputy national security adviser Yossi Draznin), and U.S. mediators from the State Department and the Pentagon [483][493]. Hezbollah refused to participate and publicly rejected the direct talks, complicating prospects because Israel insists on Hezbollah's disarmament as a precondition for a broader settlement [468][494]. Despite the extension, Israeli strikes and cross-border exchanges with Hezbollah continued during and after the talks, causing new displacement and testing the truce [517][497]. Beirut's negotiating priorities included restoration of sovereignty and Lebanese state authority, Israeli withdrawal, return of displaced persons, reconstruction, release of detainees and recovery of remains [466][469]. U.S. mediation is central to the process, while regional dynamics—notably Iran linking a Lebanon settlement to wider Iran–U.S. negotiations—add geopolitical constraints on progress [474][494]. Negotiators described the sessions as "productive," but analysts and officials warned

that ongoing violence, divergent core demands and Hezbollah's absence leave the process fragile and at risk of collapse [492][512].

Conflict Outlook

Military trends show continued low- to medium-intensity clashes along the Lebanon–Israel frontier, with Israeli incursions, airstrikes and artillery fire and reciprocal Hezbollah drone and missile attacks, and Israel reportedly studying expansion of ground operations beyond the 10-km anti-armour line [541][508][503]. Diplomatic efforts—including U.S.-hosted Lebanon–Israel talks in Washington and planned Pentagon and State Department meetings at end-May/early-June—coexist with breaches of the fragile 45-day ceasefire extension, creating a narrow window for de-escalation but a persistent risk of breakdown [533][503][517]. External dynamics, including U.S.–Iran negotiations and mediation efforts by Pakistan, Turkey and Egypt, amplify the prospect that shifts in Tehran–Washington calculations could either stabilise the Lebanese front if tied to a wider deal or trigger renewed escalation if diplomacy falters [477][577]. A negotiated settlement is possible in the short term but likely limited to temporary extensions of the truce unless the core issue of Hezbollah's weapons and Lebanese state authority is resolved—an outcome complicated by Hezbollah's rejection of direct disarmament talks and its insistence on involvement in any security decisions [531][504][532]. Humanitarian consequences are severe and likely to worsen, with displacement exceeding 1.6 million, constrained health services with repeated attacks on healthcare and suspended maternal services, and significant food assistance needs remaining despite large-scale deliveries already made [563][583][530][523]. The near-term outlook is one of continued fragility with elevated probability of localized intensification unless U.S.-brokered security mechanisms are implemented and linked to broader regional diplomacy; even with repeated temporary truces, humanitarian needs and displacement pressures will remain high [517][507][528].

Displacement

Displacement Overview

The scale of displacement in Lebanon remains very large and fluid. The Government's Ministry of Social Affairs (MoSA) registry recorded approximately 1,049,000 people self-registered as displaced as of 1 April [645]. As of 11 May, IOM reported approximately 127,210 IDPs sheltered in 628 active collective sites, while OCHA reported roughly 129,721 people in 632 collective shelters as of 14 May [645][635][651]. The vast majority of displaced households continue to live outside formal sites in host families, rented or informal arrangements. An estimated 48,500+ migrants have also been displaced [633].

Cross-border movements have been substantial: IOM reported approximately 451,000 movements from Lebanon into Syria between 2 March and 20 April, and DTM recorded 312,996 movements at Points of Entry and 115,850 individuals at arrival locations in Syria between 2 March and 11 May [633][656].

Displacement has been predominantly cross-governorate, with about 78% of IDPs relocating outside their governorate of origin. Host districts including Beirut, Aley, Chouf, Saida and Baabda together host roughly three-quarters of the displaced population [633]. The ceasefire announced in mid-April contributed to a partial slowdown in large-scale movement, but mobility remains fluid and fragile. Returns are limited or have reversed in South Lebanon, Nabatieh, the southern suburbs of Beirut and parts of the Bekaa due to ongoing insecurity, infrastructure damage and UXO contamination, prompting secondary displacement back into shelters [635][631][651]. Renewed displacement orders and airstrikes in Nabatieh governorate on 4 May triggered a further wave of displacement that pushed total internal displacement to over one million as of 11 May 2026 [665][635].

Humanitarian access remains constrained in southern governorates and Nabatieh due to damaged roads and bridges, UXO and localized insecurity, increasing pressure on collective sites and host communities. Funding shortfalls in the Lebanon Flash Appeal—approximately 42% funded as of mid-May—and the risk of pipeline breaks, particularly for health and WASH, are further constraining the scale-up of assistance [669][651].

Living Conditions of the Displaced Population

Collective sites are overcrowded and often unsuitable for prolonged habitation, as many are schools or non-residential buildings. Shelter partners have supported approximately 43,980 households with distributions totalling 125,724 mattresses, 165,598 blankets and 62,431 pillows, and are repairing or upgrading around 360 collective shelters (282 completed to date) to improve safety and privacy [669]. MoSA is scaling up site management with IOM support; IOM has facilitated referrals for thousands of displaced people in non-government-managed sites and distributed core relief items and emergency household supplies since early March [647][633][632]. Shelter monitoring has identified thousands of IDPs with disabilities in collective shelters, including approximately 1,200 with physical disabilities, 689 with mental disabilities, 422 with hearing impairments and 388 with visual impairments, underscoring the need for tailored and accessible services [667].

Attacks and damage to health infrastructure have sharply constrained services. WHO verified 158 attacks on health facilities, patients and staff—resulting in 108 deaths and 249 injuries—as of 11 May, rising to 161 incidents (110 deaths, 252 injuries) with continued facility closures by 14 May, including three hospitals and 41 primary healthcare centres operating at reduced capacity or closed [622][651]. Mobile medical teams are filling gaps: International Medical Corps reported 13,741 consultations and 35 mobile units reaching 125 shelters as of 11 May, though service continuity remains fragile and partners warn of pipeline breaks for essential medicines and supplies [622][626].

WASH partners have provided assistance in approximately 611–615 collective shelters since 2 March, delivering 88,043 hygiene kits and 50,333 menstrual kits, supplying millions of litres of potable water and installing emergency sanitation facilities. Partners flag risks of service interruptions and resource shortages if funding and access constraints persist [669][647][645].

Nutrition and maternal food security are deteriorating. Action Against Hunger and health partners report worsening infant and young child feeding (IYCF) and maternal nutrition indicators. National estimates indicate approximately 12,200 pregnant women are displaced, including 675 in collective shelters [640][667].

Protection needs remain high, with 77 protection, child protection and GBV partners reporting that they reached 192,600 IDPs by 14 May (61% inside collective sites), while approximately 390,000 children remain displaced and GBV/child protection referral capacity outside sites is limited [647]. Vulnerable groups—women and girls, older people, persons with disabilities and separated children—face elevated protection and service access gaps. Humanitarian access in southern Lebanon and Nabatieh continues to be impeded by UXO, damaged roads and bridges, and localized insecurity, compounding risks for populations in hard-to-reach areas [669][633][651][667].

Drivers of Displacement

The primary drivers of displacement in Lebanon are ongoing conflict and insecurity, environmental damage, and economic hardship. No large-scale or sustained returns have been observed in South Lebanon and Nabatieh, where Israeli-declared military zones, unexploded ordnance (UXO), damaged roads, conflict debris, and continued insecurity prevent returns and complicate humanitarian access [669][677]. Renewed airstrikes and evacuation orders in Nabatieh on 4 May exemplify how active hostilities continue to generate precautionary and forced displacement [665][635][633][660][651].

Environmental damage from earlier phases of the conflict—including extensive debris, contamination of agricultural land and water infrastructure, and tens of thousands of damaged or destroyed houses—constitutes a major structural obstacle to return and recovery [677][671]. These conditions have also contributed to secondary displacement, with households initially returning later moving again due to deteriorating conditions.

Economic pressures compound the situation: rising rental costs, increasing evictions and competition over limited services are driving housing insecurity and straining host communities, limiting both local integration and return options [655][669]. Displaced households face severe financial strain, growing reliance on borrowing and negative coping strategies, and urgent multipurpose cash needs [655][651].

Children bear acute protection and mental-health impacts from prolonged displacement. UNICEF reported at least 59 children killed or injured in a single week and estimated approximately 770,000 children experiencing heightened distress, with 72% of caregivers reporting child anxiety in a 2025 rapid assessment [682]. Worsening food insecurity is projected, with 1.24 million people expected to face Crisis (IPC Phase 3) or worse between April and August 2026 [665][638]. The Lebanon Flash Appeal was approximately 38–42% funded (roughly US\$117–129 million of US\$308 million required), and the Ministry of Social Affairs requested a three-month extension covering June–August 2026, reflecting the sustained and deepening humanitarian need [655][669][651].

Integration

The majority of displaced persons in Lebanon live outside collective shelters, in hosted, rented or informal arrangements, facing rising rental costs and increased eviction risk, particularly in Beirut and Mount Lebanon [633][655]. Movement restrictions, lack of legal residency and access constraints—including checkpoints, curfews and raids—in the North, Bekaa, Baalbek, South and Nabatieh limit displaced refugees' mobility and access to services and livelihoods [655][669]. Constrained livelihood absorption into local labour markets, combined with specific barriers for displaced households and women, reduces the effectiveness of cash-plus and training interventions where market access and productive assets are lacking [650][655].

On education, OCHA reported partial restoration of services with over 800 public schools reopened for approximately 220,000 students. As of 14 May, approximately 164,059 displaced children were accessing formal online education and 94,067 were attending in-person classes [669][647]. The Ministry of Education and Higher Education (MEHE) has authorized non-national students to register for national exams regardless of residency status, reflecting active institutional efforts to maintain access despite a strained environment [647].

Cash support has been scaled up but remains insufficient. The Government-led Shock Responsive Social Safety Net disbursed approximately USD 15.6 million to 138,000 households [669]. The 2026 Lebanon Flash Appeal had received approximately US\$129 million of US\$308 million required (approximately 42% funded) as of mid-May, constraining longer-term integration and service expansion [651]. Health and WASH services are being sustained via mobile medical teams, primary health centres and shelter-based WASH interventions, but continuity is threatened by access constraints and funding shortfalls [647][651]. MoSA is leading site-management scale-up with IOM support, while UNHCR co-leads the protection and shelter sectors with MoSA, reflecting active institutional coordination in a challenging operating environment [647][655].

Humanitarian Access

Humanitarian Access Overview

Humanitarian partners, coordinated with national authorities and supported by the Humanitarian Notification System (HNS), maintain operations and reach conflict-affected communities across southern Lebanon and other governorates including Beirut (Dahieh), Mount Lebanon, Bekaa, the North, Baalbek, Nabatieh, Tyre and Marjayoun [700][713][698]. Key actors active in delivery include UN agencies and coordination bodies (OCHA, WHO, IOM, UNHCR), international NGOs (e.g., International Medical Corps, ACF), local partners and government ministries, with bilateral support via the EU Humanitarian Air Bridge and the Government of Luxembourg for airlifts [701][708][704][703][698]. Modes of delivery include HNS-facilitated land movements, emergency airlifts (over 4,500 shelter and household items airlifted on 11 May), mobile medical teams, water trucking and pre-positioning of supplies [713][698][704][700]. Despite these efforts, sustainability and service continuity remain at high risk without urgent funding and safer access conditions, as partners continue to adapt response modalities to reach hard-to-reach and newly displaced populations [700][690][698].

Barriers to Humanitarian Access

Humanitarian access in Lebanon is increasingly constrained by active hostilities, an Israeli-declared military zone in the south, movement restrictions (checkpoints, curfews, raids), and widespread infrastructure damage – including damaged roads and bridges, conflict debris, and unexploded ordnance (UXO) contamination – with the South and Nabatieh governorates especially affected as of mid-May 2026 [713][690][698]. These conditions have delayed and limited aid deliveries despite coordinated facilitation: the HNS enabled 121–131 facilitated movements to communities across southern Lebanon between 2 March and mid-May [713][698]. Repeated attacks on healthcare have directly impeded medical response; since 2 March, at least 161 incidents affecting healthcare have been recorded, resulting in approximately 110 deaths and 252 injuries among health personnel, with multiple hospitals and primary healthcare centres remaining closed [698][690]. Operational capacity is further strained by displacement of national staff, fragmented referral pathways for secondary and specialized care, administrative and bureaucratic impediments, and critical funding shortfalls – the 2026 Lebanon Flash Appeal was approximately 42% funded (around US\$129 million of US\$308 million required) as of mid-May 2026 [713][700][704][690][698]. Service sustainability and supply-chain risks threaten continuity of health, WASH and food assistance, with partners warning of potential WASH interruptions in collective shelters and pipeline breaks if conditions persist [698][704].

Humanitarian Access Gaps

As of 11 May 2026, IOM's Displacement Tracking Matrix (DTM) reported over one million internally displaced persons in Lebanon, including 121,701 people sheltering across 628 collective sites, while the majority remain outside formal sites in host communities, rented accommodation or informal arrangements [722]. Access constraints are concentrated in southern Lebanon and Nabatieh governorates, where UXO contamination, damaged roads and bridges, conflict debris and localized insecurity are delaying humanitarian movements, hindering restoration of services and restricting access to return areas and other hard-to-reach locations [713][722]. People living in tents, public spaces or other open areas – particularly women and girls, persons with disabilities and approximately 390,000 displaced children – face the highest protection and basic-service deficits, including urgent needs for safe accommodation, water and sanitation, and tailored support [700]. Key sectoral gaps identified between 5–14 May include overstretched shelter capacity in collective sites and host communities, disrupted continuity of healthcare with limited access to secondary and specialized services, and gaps in protection, GBV and child-protection referral pathways and services, all exacerbated by fragmented systems and insufficient funding [713][700]. Regarding logistics, the Masnaa border crossing remained open and operational and humanitarian cargo is exempted from transshipment requirements, but overall mobility and logistics remain fragile [722].

Information, Communication and AAP

Information Landscape

Coordination is led by the Ministry of Social Affairs under the Lebanon Response Plan, with UNHCR co-leading the Inter-Sector Coordination Group and co-leading the Protection, Shelter/Core Relief Items and Cash Working Group alongside government and NGO partners, providing the primary structure for information collection and sharing [779]. Information systems are producing actionable outputs – assessment reports, rapid health findings, and logistics reporting – though operational capacity and funding gaps constrain consistency and scale. The Lebanon Flash Appeal targets USD 308.3 million to assist 1 million people but was only approximately 38–42% funded as of mid-May, limiting partners' ability to sustain monitoring and response operations [779][801][757][766].

Active, time-bound needs assessments are underway across sectors. UNHCR completed field data collection for a nationwide GBV assessment (37 focus group discussions and 21 key informant interviews), with data collection closed on 8 May and publication expected by end-May [779]. WHO reported that approximately 85% of the Rapid Health Assessment had been completed as of 14 May 2026, with preliminary findings to inform Ministry of Public Health decision-making [791]. International Medical Corps reported routine monitoring of medical activity and mobile outreach, including 13,741 consultations and 35 mobile units reaching 125 shelters as of 11 May [781]. IOM/DTM produce regular mobility snapshots and rapid assessments – consulting 6,057 key informants across 1,229 locations as of 11 May – to inform operational planning [812][770].

The Logistics Cluster publishes live convoy schedules weekly, provides common storage and cargo consolidation services, and uses the Log.ie platform to share real-time logistics constraints and telecommunications connectivity information with partners [769][763]. Key operational figures as of mid-May include:

- Protection actors reached 182,313 IDPs (62% inside collective sites), delivering MHPSS to 64,976 people, 1,259 case management services, 1,796 protection cash grants, and 24,841 dignity kits [769].
- Shelter partners supported 44,106 households and distributed 127,028 mattresses, 166,816 blankets, and 67,054 pillows as of 14 May [763].
- The Logistics Cluster supported 49–51 partners, facilitated 22 convoys to hard-to-reach areas, offloaded cargo across 16 municipalities, and ran a regional convoy from Amman on 7 May to reduce lead times [769][763].

Communication Channels

The Humanitarian Notification System (HNS) serves as the primary coordinated channel for civil-military deconfliction, facilitating 131 approved humanitarian movements between 2 March and 14 May to deliver life-saving supplies across southern Lebanon and other affected locations [763][757]. The Logistics Cluster's regional convoy service – including the first Amman–Lebanon convoy on 7 May – further supports timely supply delivery, with real-time logistics and telecommunications data shared via the Log.ie platform [769][763].

Digital platforms support information exchange and capacity-building: WHO delivered online training and a radio-nuclear hazards webinar on 15 May, and printed RCCE materials and workshops were implemented to support public health messaging [791][761]. Face-to-face channels remain central, with mobile medical teams, field visits, shelter-based workshops, and site management activities delivering services, referrals, and community engagement across collective shelters and host communities [781][762][761][775]. Community-based outreach through 19 of 21 Community Development Centres continued operations, reaching displaced people with service information and two-way feedback mechanisms [779][769].

Returnees and displaced people often rely on phone calls, social media, and short "go-and-see" visits for situational updates. While these sources tend to provide more reliable security information, they offer limited granular detail on services such as health or market availability, creating information gaps that affect return and assistance decisions [778]. Telecommunications and mobile networks have been repeatedly degraded by outages, GPS spoofing, and route unpredictability, reducing reliability for both responders and affected populations and complicating real-time coordination [763][769][791]. Communications disruptions and rapidly changing route conditions force last-minute operational adaptations, reducing the timeliness and predictability of aid deliveries [763][769].

Barriers to Information

Physical access constraints are a primary barrier to information delivery. Damage to infrastructure, localized insecurity, unexploded ordnance contamination, and movement restrictions in the South and Nabatieh governorates are constraining humanitarian movements and hindering the delivery of life-saving information alongside supplies [765][757][759]. Reduced humanitarian presence at border points and limited capacity at arrival locations have further curtailed immediate assistance and information services for returnees and newly arrived people [771][812].

A broader energy crisis is impairing telecommunications and other critical infrastructure, increasing the likelihood of communication blackouts that would limit information flow to affected populations [800]. Telecommunications connectivity assessments were finalized and indicate risks to operational continuity from power outages and connectivity disruptions, with initial funding planned to cover only three months while additional resources are being mobilized [763].

Specific population groups face compounding barriers. Migrants and non-Arabic speakers encounter language barriers and concerns over migration status that restrict their access to shelter, assistance, and reliable information, limiting their ability to request help or evacuation assistance [775]. Education disruptions and limited access to remote learning exacerbate information gaps for children and caregivers, particularly in active conflict zones and displacement settings [838]. Protection outreach has relied heavily on one-way mass messaging and hotlines, while reach outside collective shelters remains limited at around 39%, indicating uneven access to two-way information and complaints mechanisms [769][771].

Underfunding of the 2026 Lebanon Flash Appeal – approximately 42% funded as of mid-May – and critical funding shortfalls in the health sector are reducing partners' capacity to sustain outreach, risk communication, and community engagement activities [757][784][766].

Misinformation and Rumors

Misinformation and polarized narratives increased public anxiety following the ceasefire announcement, contributing to short-lived returns and reversed displacement movements as of 11 May [769]. Social media and the rapid sharing of maps, videos, and alerts have circulated both authentic and out-of-context content, prompting warnings that unverified posts can mislead residents about local risk zones [780]. Misinformation has compounded displacement pressures and shelter demand amid renewed displacement orders and fluid movement patterns observed across governorates [770][769]. National agency short dispatches are being used as baseline information, but authorities and responders note these require field confirmation to avoid amplifying miscontextualized reports [780].

In response, humanitarian actors have deployed mass communication and structured feedback channels. Protection partners disseminated one-way protection messages reaching approximately 90,000 people and processed about 21,000 protection feedback and complaints via hotlines as of 11 May [769]. WHO is developing an all-hazard Risk Communication and Community Engagement (RCCE) and infodemic management strategy for Lebanon, and deployed a regional consultant in April to establish a multisectoral MHPSS operational framework, with missions underway to strengthen safeguarding and GBV/PRSEAH integration as of 14 May [761][791]. The Emergency Preparedness Working Group convened a thematic session on communication products, referral systems, and hotlines, though gaps persist in multilingual harmonization – notably in Farsi – and alignment of referral mechanisms [771].

Accountability Needs and Transparency

Community-based accountability mechanisms are operational but unevenly accessible. Nineteen of 21 Community Development Centres remained active, providing outreach, service information, and feedback and complaint mechanisms, reaching 9,724 displaced people (63% women and girls) since March [779]. Protection, child protection, and GBV partners reported reaching 192,600 internally displaced persons, with community outreach and information sessions reaching 89,651 people on protection, safety, and access to services [763]. Emergency Rapid Needs Assessments and other data collection processes are being used to inform response prioritization and planning [769].

The Health Sector and WHO are scaling up RCCE efforts, including a Lebanon case study on integrated NCD and MHPSS community engagement to improve awareness and participation [766][761]. However, systemic gaps persist: a formative Global Health Cluster evaluation found that accountability to affected populations remains weak, with few mechanisms enabling affected people to meaningfully influence decision-making [754]. Protection monitoring similarly reports that complaints and feedback mechanisms remain unevenly accessible, particularly for marginalized groups outside collective shelters [755].

Ongoing insecurity, movement restrictions, and infrastructure damage — compounded by significant funding shortfalls, with the 2026 Flash Appeal approximately 42% funded as of mid-May — are constraining partners' ability to sustain and scale accountability mechanisms and consistent engagement with affected communities [763][757].

Legal and Policy

Legal and Policy Framework

Despite the ceasefire that started on 17 April, Israeli airstrikes, demolitions and renewed displacement orders continued across southern Lebanon, undermining humanitarian access and prospects for safe returns [873][852]. UN agencies and the Humanitarian Coordinator reiterated that parties are bound by international humanitarian law to protect civilians, medical personnel and humanitarian infrastructure and to facilitate rapid, unimpeded humanitarian passage [873][857]. Lebanon has sought to consolidate the ceasefire into a more sustainable agreement and requested US pressure on Israel to stop attacks and demolitions during Washington-hosted talks [876]. A draft declaration of intent circulated for Lebanon-Israel negotiations foresees Israeli withdrawal from Lebanese territory, handover of occupied areas to the Lebanese Armed Forces (LAF), LAF deployment with US/international training and support, and references UNIFIL's mandate ending on 31 December 2026 [895][891]. The Lebanese National Council for Scientific Research published a report covering October 2023–December 2024 estimating about US\$25 billion in damage, and the Lebanese Minister of the Environment formally accused Israel of ecocide [904][906]. Commentators and experts have invoked international accountability mechanisms, noting ICC arrest warrants issued in November 2024 and ICJ interim measures from January 2024 in discussions on alleged violations and accountability [930]. Humanitarian coordination mechanisms — including OCHA-led inter-sector coordination, the Lebanon Response Plan/Flash Appeal and the Humanitarian Notification System — continue to prioritise response planning and to facilitate humanitarian movements to hard-to-reach areas despite access constraints and funding shortfalls [864][852][857].

Humanitarian Impact of Legislation

The Ministry of Social Affairs (MoSA) leads the Lebanon Response Plan, while UNHCR co-leads multiple sectors — including protection, shelter/core relief items and cash — and retains overall responsibility for the refugee response, embedding humanitarian coordination within government-led frameworks [851]. Lebanon's main social protection instrument, the National Poverty Targeting Programme administered by MoSA, was expanded after 2019 to deliver direct cash support and remains the primary formal channel shaping access to assistance, while local political affiliation and fragmented systems continue to influence entitlements and distribution [859][865]. Movement restrictions — checkpoints, curfews and raids — together with lack of legal residency are reported to constrain mobility for displaced refugees and Lebanese in the North, Bekaa, Baalbek and parts of the South, increasing eviction and access risks [851][867]. Arrivals and returnees frequently face missing, damaged or lost personal IDs, residency papers and family records, which hinder access to services and require administrative and legal support to replace [883][867]. Border control measures further affect cross-border assistance and population movements: Masnaa and Al Qaa remain operational, Aarida is partially open to pedestrians only, Abboudiyeh's reopening was postponed until 1 July, and Jisr Qmar/Bouqaia was only partially and unofficially reopened for limited pedestrian movement [866]. Humanitarian access is increasingly constrained by declared military zones, infrastructure damage, unexploded ordnance contamination, GPS spoofing and rapidly changing route conditions; the Humanitarian Notification System has facilitated over 100 coordinated movements to deliver aid since March, but operational windows remain narrow and unpredictable [857][852]. Attacks on healthcare and medical personnel have been repeatedly recorded since 2 March, undermining protection of medical services and prompting UN calls for respect for international humanitarian law and unimpeded humanitarian passage [864][852][873].

International Agreements Impacting the Crisis

US-mediated direct Lebanon-Israel negotiations convened in Washington in mid-May, with the Lebanese delegation led by Simon Karam (joined by Ambassador Nada Moawad and military attaché Brig. Gen. Oliver Hakmeh) and an Israeli delegation headed by Ambassador Yechiel Leiter and military representatives [882][856][907]. Lebanon framed its negotiating priorities as an effective ceasefire, full Israeli withdrawal, return of displaced persons, release of prisoners, deployment of the Lebanese Armed Forces and reconstruction, while Israel's agenda prioritized neutralizing Hezbollah and securing border protections [870][850][876][907]. The ceasefire that began on 17 April remained fragile, with Lebanese authorities reporting cumulative fatalities in the mid- to high-2,800s and recurrent violations during the Washington talks [873][858][849]. UNIFIL and the prospect of expanded Lebanese Armed Forces deployment south of the Litani were central to proposed implementation arrangements, with Lebanon presenting legal texts and UN resolutions — including Resolution 1701 — to underpin any stabilization package [856][907][876]. UN Humanitarian Coordinator Imran Riza and UN agencies repeatedly called for protection of civilians, respect for international humanitarian law and rapid, unimpeded humanitarian access during the negotiations [873][853]. On the humanitarian side, an EU/Luxembourg airlift delivered more than 4,500 emergency relief items on 11 May, and partners reported large-scale food and shelter assistance including over 9.2 million meals distributed, even as funding and access constraints limited further scale-up [878][857]. The UN's \$308 million Lebanon emergency appeal (March–May 2026) was reported approximately 41–42% funded as of mid-May, constraining longer-term government-led response and recovery planning [903][857][868].

Limitations of Current Policies

Humanitarian access and civilian protection in Lebanon are being undermined by sustained attacks on health services, widespread infrastructure damage and route insecurity, concentrated access constraints in southern Lebanon (South and Nabatieh), fragmented referral systems, and severe funding shortfalls that together reduce service continuity and operational adaptability [864][852][857][878][868].

Key figures:

- 161 recorded incidents affecting healthcare, resulting in 110 deaths and 252 injuries among health workers (reported as of 14 May 2026) [868][857].
- Over 1,049,000 internally displaced people reported by the Ministry of Social Affairs (as of 01 April 2026) [855].
- Approximately 129,721 people sheltering in 632 collective shelters (reported as of 14 May 2026) [868].
- The 2026 Lebanon Flash Appeal was approximately 42% funded as of mid-May (US\$129 million of US\$308 million required), limiting partners' ability to sustain and scale responses [857][868].
- At least 131 humanitarian movements carrying essential supplies were facilitated through the Humanitarian Notification System since the escalation began, illustrating reliance on negotiated corridors with limited capacity [857][868].

Priority policy reforms indicated by operational gaps include: rapidly scaled, flexible funding allocations to prevent pipeline breaks in health and WASH and to meet urgent sector shortfalls — the Health Sector called for immediate replenishment and an additional US\$37 million to sustain services [874][857][868]; strengthened enforcement and accountability measures to protect medical personnel and infrastructure and to ensure respect for international humanitarian law in practice [864][868][873]; targeted investments in route restoration, unexploded ordnance clearance and civil-military coordination to restore reliable humanitarian access to hard-hit southern and Nabatieh areas [852][857][864]; reform of fragmented referral pathways and more decentralized coordination with municipalities to maintain continuity of secondary and specialized care and to support safe, dignified returns [852][868]; and measures to reduce legal vulnerability for migrants and undocumented groups through documentation and legal-status support and inclusive access to aid, removing barriers to assistance [857][889].

Food Security and Livelihoods

Access to Food and Market Functionality

Acute food insecurity in Lebanon is projected to affect approximately 1.24 million people between April and August 2026, up from about 874,000 in the November 2025–March 2026 period, driven by escalating hostilities, mass displacement, income losses and rising costs [945][959]. Markets remain operational in several cities but high prices, transport disruptions and local breakdowns are preventing many households from buying essential food, while currency depreciation and fuel price increases have eroded purchasing power and pushed food prices higher [945]. WHO and health partners note that food affordability is the principal constraint and that nutrition risks are rising—particularly for infants, young children, pregnant and lactating women—amid limited dietary diversity and reduced meal frequency in displaced and host households [959][951].

Growing access constraints, infrastructure damage and insecurity—particularly in South and Nabatieh governorates—are disrupting supply chains and humanitarian operations, prompting partners to preposition food and adapt delivery modalities to sustain assistance in hard-to-reach areas [958][948][970]. The food security sector is engaging with authorities on measures such as the Government-led Shock Responsive Social Safety Net (SRSN), but sustained funding shortfalls and operational constraints are limiting the scale and sustainability of responses [970].

Key figures:

- +1,049,000 IDPs reported by the Ministry of Social Affairs (as of 01 April) [958].
- 127,210 IDPs in collective sites (as of 11 May) [958].
- Food assistance delivered since the escalation: 8.5 million hot/cold meals and 108,563 ready-to-eat (RTE) kits (reported as of 11 May) [948].
- Reported totals by 14 May: over 9.2 million hot/cold meals and 120,936 RTE rations distributed [970].

Coping Mechanisms

Households across Lebanon are reducing meal quality and frequency, skipping meals, delaying care, selling goods and exhausting savings, and increasingly depending on humanitarian assistance [945][948]. Displaced and host households report growing reliance on borrowing and accumulating debt as a coping strategy, reinforcing cyclical financial vulnerability [972][971]. Evaluations of cash and cash-plus programming in Lebanon indicate that cash often provides short-term relief but does not remove underlying debt cycles or secure durable livelihoods without complementary market access and sustained support [971].

Protection actors report rising use of negative and harmful coping mechanisms, including increased risks of child labour, and note that roughly 390,000 children remain displaced, heightening child protection concerns [970][948]. Some southern districts—Bent Jbeil, Marjayoun, Nabatiyah and Tyre—show 55–65% of the population at high food-insecurity levels, with about 10% in the emergency phase [945][967]. Humanitarian partners have delivered large-scale food assistance and protection cash grants, but coverage gaps and funding shortfalls persist, limiting the reach and sustainability of support [948][970].

Livelihood Disruption

Escalation of hostilities and economic shock have driven widespread job losses and business closures in Lebanon, with Economy Minister Amer Bisat estimating an economic loss of around 7% of GDP as companies close and people lose work [978]. Hospitality and service businesses report catastrophic revenue declines (one restaurateur cited a ~90% drop), while traders have closed and many employees and daily workers cannot reach workplaces, eliminating regular income for large numbers of households [978][945]. High fuel and power costs, plus generator and transport expense increases, are raising operating costs and reducing hours and productivity for businesses and informal workers [978][970].

Displacement of approximately 1.2 million people has reduced household income streams as families shelter in schools or rent out properties without replacement earnings, with savings exhausted and growing reliance on borrowing and negative coping strategies [978][945][972]. According to IOM's Displacement Tracking Matrix (DTM, 11 May 2026), returnees and arrivals face very limited and inadequate income: 57% of assessed arrival locations reported returnees not engaged in any employment; where work exists it is largely informal, with daily wage labour (75%), salaried employment (39%) and agriculture/livestock (38%) the most cited sources; most key informants judged income insufficient (51% not sufficient at all; 47% only partially sufficient), and remittances (19%) and humanitarian assistance (5%) covered only a minority of locations [955][973].

The 2026 tourism season has effectively collapsed, removing a major source of seasonal and informal employment—including drivers, cleaners, suppliers, guides, artisans and other short-term workers—and threatening permanent closure of small hotels, guesthouses and related service providers [974].

Impact on Agricultural Activities and Production

Assessments estimate direct damage to Lebanon's agricultural sector at USD 118 million, with losses of USD 586 million (total damage and losses exceeding USD 700 million) and reconstruction and recovery needs of USD 263 million, of which USD 95 million are prioritised for 2025–2026 (Ministry of Agriculture / UN assessments) [982]. The Ministry of Agriculture has estimated some 54,000 hectares of agricultural land directly affected (about 22.5% of the country's 250,000 ha of agricultural land), while UNDP identified more than 12,000 hectares of abandoned agricultural land in Nabatiyah and southern Lebanon, and roughly 76% of farmers have been displaced [982][988].

Irrigation and water-collection infrastructure have been damaged, including networks linked to the Litani Authority and the destruction of about 20 local water collection basins and their associated networks, constraining irrigation-dependent production and recovery [982]. Assessments also report the destruction of agricultural support infrastructure—about 23 cooperatives, 16 olive mills and over 200 nurseries—alongside crop and orchard losses including 134 ha of olive groves, 48 ha of citrus, 44 ha of banana and 15 ha of other fruit trees [982]. Beekeeping and livestock were directly affected: an estimated 1,500 hives destroyed, about 10% of animals lost and a further 30% sold at reduced prices in southern governorates, eroding productive capital and slowing recovery prospects for households [982].

Field reports indicate greenhouses have been damaged, agricultural roads are dangerous, and many farmers have abandoned their land, limiting access for planting, tending and harvesting and threatening upcoming seasons [945][982]. Restoration and return to production are further constrained by damage to small- and medium-scale irrigation installations and the need for parts, fuel, technicians and secure access to repair and operate pumps and networked systems, undermining prospects for timely recovery of livelihoods and seasonal crops [982].

Food and Livelihood Needs and Current Response

Since the escalation on 2 March 2026, humanitarian partners have delivered large-scale in-kind food assistance, reporting over 9.2 million hot and cold meals and 120,936 ready-to-eat (RTE) rations as of 14 May 2026 [970][948]. Assistance modalities in use include hot/cold meal distributions, RTE kits and food parcels, multipurpose cash assistance, targeted protection cash grants (1,796 grants reported), pre-positioning of supplies, and a planned self-cooking pilot in 10 shelters in the first week of June 2026 [970][948]. Coverage outside collective shelters remains limited: protection actors note only 39% of people reached to date were supported outside formal sites, indicating underserved populations in host-community and informal settings [972].

Nutrition actors report substantive coverage gaps, with 38% of collective shelters lacking nutrition support and major gaps concentrated in Beirut and Mount Lebanon; ACF screening and outreach reached 404 children and pregnant/lactating women and 732 people through awareness sessions, while isolated sites (Rmeish, Ain Ebel) have at least 200 children aged 0–3 identified as needing nutrition support [949][951]. Livelihoods and cash-plus programming provide short-term stabilisation but face limits: evaluations from Lebanon show cash eased immediate consumption needs yet rarely translated into sustained income without integrated market-linked support and post-training income assistance, while labour-market constraints meant many training placements did not yield continued employment [971].

Operational constraints and funding shortfalls are limiting scale and sustainability, with insecurity, movement restrictions and infrastructure damage impeding reach and the 2026 Lebanon Flash Appeal approximately 42% funded (around US\$129 million of US\$308 million) as of mid-May 2026 [970][944]. Coordination mechanisms are active: sectors are engaging with the Ministry of Social Affairs on a possible government-led Shock Responsive Social Safety Net, using the Humanitarian Notification System to facilitate humanitarian movements, and consulting the Food Safety Task Force to operationalise shelter self-cooking and approvals [970].

Education

Learning Environment and Facility Conditions

As of 11 May 2026, 448 educational institutions (332 public, 67 private, 49 TVET) remained unavailable for educational use because they were sheltering internally displaced persons [1013]. Hundreds of schools in southern Lebanon have been destroyed or repurposed as collective shelters, with UNESCO estimating 339 schools in warzones and hundreds converted to shelters, intensifying disruptions to learning since March 2026 [1010]. More than 800 public schools had reopened for over 220,000 students, about 100 operated in hybrid mode serving approximately 27,000 students, and over 580 public schools were delivering remote learning to more than 116,000 students, including 320+ in conflict-affected areas [1013]. A significant participation gap remained for afternoon-shift students, with only 23% of enrolled students in remote afternoon learning actively using the platform, partially because national education broadcasts run 07:30–14:00 and exclude the afternoon shift [1013][1011]. Teacher capacity is strained: salaries have been devalued by about 80% and roughly 30% of the sector left the country or changed professions since 2019, with many teachers further displaced by recent hostilities [1010]. Continuity of online learning remains constrained by poor connectivity and limited device access, and education-in-emergencies measures include distribution of education kits, approval for partners to run learning activities inside shelters, and MHPSS and staff well-being training for frontline education responders [1011][1013].

Access to Education and Barriers

Since March, Israeli attacks have displaced more than 1.2 million people in Lebanon, including an estimated 500,000 school-aged children, with many families remaining in collective shelters (approximately 125,000 people, including roughly 44,800 children) [1010][1031][1014]. According to UNESCO, about 100 schools are in high-risk areas that could soon become inaccessible, on top of the hundreds already converted to shelters or located in active warzones [1010]. Key barriers to enrollment and regular attendance include poor connectivity, intermittent electricity and lack of devices for remote learning, widespread use of school buildings as shelters, ongoing insecurity and movement restrictions (checkpoints, curfews), and acute affordability pressures that force families to choose between basic needs and education — contributing to rising dropouts, child labour, and early marriage as harmful coping mechanisms [1010][1014][1031]. Psychosocial distress and learning loss are widespread, with UNICEF estimating some 770,000 children experiencing increased psychological distress and reporting that, since the ceasefire, dozens of children have been killed or injured — factors that further hinder return to school and sustained attendance [1016]. MEHE has prioritized in-person learning, approved partners to deliver learning activities within shelters, maintained registration for national examinations (open until 15 May), and confirmed that Syrian and other non-national students may register for intermediate and secondary certificate examinations upon submission of certified educational documents [1011][1013].

The tutoring hotline (Call&Learn) recorded a sharp increase in use – approximately 13,018 children accessed it since 1 May, a 400% increase since March 2026 – reflecting high demand for remedial support [1013].

Education Needs and Current Response

By 14 May 2026, approximately 164,059 internally displaced children had accessed formal education online, 94,067 attended in-person classes, and 4,222 were enrolled in non-formal education programmes [1011]. The Ministry of Education and Higher Education (MEHE) activated online accounts for 390,000 students and 40,000 teachers to support continuity of learning [1013]. Education partners distributed 1,694 education kits reaching roughly 84,700 children, provided 764 individual student kits and learning materials, and supported 49 children with disabilities with assistive devices and/or rehabilitation and therapy services [1011]. Partners are also delivering targeted trainings for frontline staff on well-being, inclusive approaches, psychosocial support, and referral pathways, while UNESCO, UNICEF, and UN/sector partners have promoted measures such as opening multiple shifts, establishing temporary learning centres, integrating psychosocial and mental health services, and scaling safe spaces and community-based MHPSS to mitigate learning loss and trauma [1011][1010][1016][1014]. Key gaps include the large number of schools still used as shelters (limiting access to safe, functional classrooms), pronounced participation shortfalls for afternoon shifts, limited connectivity and devices, insufficient learning hubs and teacher training, and inadequate coverage in hard-to-reach areas; continued donor support is needed to scale up learning hubs, trainings, and distribution of learning materials [1011][1013].

Health

Availability and Access to Healthcare Services

Health service availability remains severely constrained, with three hospitals and 41–42 primary health care centres (PHCCs) closed, six PHCCs providing only restricted (chronic/emergency) services, and six hospitals yet to resume delivery services – limiting maternal care in Tyre, Bint Jbeil, Hasbaya and Nabatieh for almost two months [1066][1076][1069][1071]. Damaged infrastructure, limited health staffing, gaps in essential medicines and reduced maternal and emergency services are overburdening remaining functional facilities [1067][1069]. Humanitarian access and referral pathways are increasingly constrained by UXO contamination, damaged roads and bridges, conflict debris and localised insecurity, complicating patient transport and secondary referrals; HNS-facilitated movements (121 movements to 11 May) and IOM transport support (1,766 people transported to PHCs) have been used to reach hard-to-reach areas [1072][1084][1074].

Outreach and mobile modalities are sustaining access where possible. Partners report 211 outreach units linked to 185 PHCCs, and 35 mobile medical teams reached 125 shelters and community sites delivering 13,741 consultations and other services as of mid-May [1076][1073][1080]. Maternal and child services remain disrupted: outreach recorded 4,756 antenatal consultations, but deliveries have decreased at some hospitals (e.g., Baalbek Governmental Hospital), and displaced pregnant women in shelters face suboptimal meal frequency; an estimated 12,200 pregnant women are displaced nationwide based on March–May assessments [1076][1066][1085][1070].

A Rapid Health Sector Partners Capacity Assessment conducted in late April found 11% of partners at high risk and 53% at moderate risk of service interruptions within 4–8 weeks absent new funding, underscoring the fragility of service continuity [1066]. The Health Sector and MoPH have called for urgent replenishment of essential medicines, trauma kits and EmONC items and requested an additional USD 37 million to avert further deterioration, while WHO's regional emergency appeal remains critically underfunded at approximately 13% [1066][1070].

Disease Outbreaks

Displacement, overcrowding in collective shelters, damaged sanitation infrastructure and disrupted WASH services are increasing the risk of communicable disease transmission across Lebanon [1072][1085][1070]. Field assessments report high prevalence of acute diarrhoea, respiratory infections, eye infections and skin conditions among infants and young children in shelters, driven by damp, unhygienic and overcrowded living conditions [1085]. No major disease outbreaks were reported in shelters as of mid-May, though partners note intermittent food-poisoning incidents and persistent outbreak risk in overcrowded sites with compromised WASH and limited services [1070]. Regional outbreaks – notably measles and hepatitis A in neighbouring Syria – linked to immunity gaps, population movement and overcrowding represent additional cross-border risk that WHO notes could affect Lebanon's vulnerable displaced populations [1067].

Health authorities and the Health Sector have called for expanded facility- and community-based surveillance, strengthened risk communication and rapid response capacity. WHO and partners are supporting surveillance, rapid response teams and risk communication activities, while Rapid Health Assessment data are being finalised (approximately 85% complete by mid-May) to guide response planning [1066][1069]. Vaccination and outreach efforts are ongoing: MoPH-linked outreach through 211 primary outreach units reported 9,840 IDP children vaccinated and 296,215 primary care consultations, with 110,069 IDPs receiving medications and 48,851 supported for chronic medications as of 14 May 2026 [1076]. IOM and partners also report 10,651 TB screenings and 14,614 health-awareness contacts in displacement settings to support early detection of communicable threats [1084]. Mobile medical teams continue routine vaccination and service delivery at shelters, with International Medical Corps reporting 262 routine vaccinations across 35 mobile units reaching 125 sites by mid-May [1073][1080].

WASH partners have scaled shelter support – including distribution of hygiene and menstrual kits and millions of litres of potable water – but critical sanitation gaps and funding shortfalls limit the ability to fully mitigate public-health risks in collective sites [1072][1077]. The health system's capacity to prevent and contain outbreaks is under severe strain, with multiple hospitals and roughly 41–42 PHCs closed or partially functional and shortages of essential medicines and diagnostics constraining outbreak response [1066][1069][1076].

Health System Challenges

Health service delivery remained severely constrained in mid-May 2026, with damaged health infrastructure, limited staffing and ongoing insecurity restricting access to care and increasing pressure on functional facilities [1067]. Multiple hospitals sustained physical damage (16 establishments reported), and dozens of PHCCs, pharmacies and pharmaceutical warehouses were damaged or closed, further degrading service capacity and supply storage [1069][1087].

Attacks on healthcare have continued despite the ceasefire: since the onset of hostilities on 2 March, the Health Sector recorded 155–161 attacks on health care resulting in 104–110 deaths and 244–252 injuries among health-care workers, with continued post-ceasefire incidents including the killing of paramedics on 12 May [1066][1076][1103][1072]. Health partners reported critical shortages of trauma and surgical supplies, blood bank capacity, trained personnel, essential medicines including chronic disease treatments, diagnostics and emergency equipment [1067][1069][1070]. Humanitarian access and referral pathways were further constrained by UXO contamination, damaged roads and bridges, and localised insecurity, delaying secondary and specialised referrals to functioning facilities [1074][1072].

Funding shortfalls are acute: the 2026 Lebanon Flash Appeal was approximately 42% funded (roughly USD 129 million of USD 308 million required) as of mid-May, and WHO's USD 30.3 million emergency appeal remained critically underfunded at approximately 13%, limiting partners' capacity to maintain and scale essential health and WASH services [1077][1070]. WHO and partners are supporting 14 emergency medical teams (36 deployed personnel) across 21 frontline hospitals, prepositioning trauma and emergency surgery kits and ICU ventilators, and providing NCD medication substitution guidance and trainings to sustain continuity of care [1067][1069].

Mental Health and Psychosocial Support (MHPSS)

Protection and health partners have scaled MHPSS delivery significantly in mid-May 2026. Sector figures indicate 64,976 people participated in MHPSS interventions as of 11 May, rising to 68,528 as of 14 May; UNHCR provided MHPSS to 4,971 individuals during 4–10 May, and IOM reached 4,159 people with MHPSS since 2 March [1072][1076][1079][1084]. WHO has supported inpatient care for severe mental health conditions for 38 patients since 2 March (34 Lebanese, 2 Syrian refugees, 2 migrant workers) and co-chairs the MHPSS Coordination Group with the Ministry of Public Health/National Mental Health Programme [1069].

Community-based MHPSS is provided through Community Development Centres (CDCs), collective shelters and outreach by trained social workers, counsellors and psychologists — 19 of 21 CDCs remained operational and CDCs have reached 9,724 displaced people since 2 March [1079]. The National Mental Health Programme and partners are delivering self-care and burnout-prevention trainings for frontline workers across governorates in May (eight sessions), and WHO conducted a mental health professional training on 13 May for 110 participants with another session scheduled for 16 May [1066][1067].

Children and other vulnerable groups face acute mental health needs: UNICEF estimates approximately 770,000 children are experiencing increased psychological distress; partners reported 978 persons with disabilities reached with specialised support; and shelter monitoring records include 689 IDPs reported with mental disabilities in collective sites [1065][1068][1076][1069]. Hotline and remote support mechanisms remain active, including the Ministry of Public Health mental health support hotline (1564), a national suicide prevention hotline, and NGO-operated free emotional-support hotlines such as the IRC's "National Lifeline" [1069][1066][1092]. Despite these efforts, agencies warn that needs far outstrip available resources and that staffing shortages and funding gaps risk service interruptions, limiting the ability to reach all people in need [1068][1070][1076][1077].

Health Needs and Current Response

The Health Sector urgently calls for protection of medical personnel and rapid, flexible funding. An additional USD 37 million is required under the Lebanon Flash Appeal (March–May 2026) to sustain emergency health operations and prevent further deterioration of public health outcomes [1066]. The Flash Appeal was approximately 42% funded as of mid-May, and WHO's USD 30.3 million emergency appeal remained critically underfunded at approximately 13%, constraining the ability to replenish supplies, scale trauma and maternal services, and prevent pipeline breaks [1070][1077].

Primary care outreach is being maintained at scale: 211 outreach units linked to 185 PHCCs reported 296,215 consultations, 110,069 IDPs receiving medications, support to 48,851 chronic patients, 9,840 child vaccinations and 4,756 antenatal consultations as of 11–14 May [1076]. International Medical Corps mobile teams (35 units) reached 125 shelters and community sites, providing 13,741 consultations, 7,213 acute treatments, 1,596 chronic disease services, 262 routine vaccinations, nutrition screening and MHPSS support [1073][1080]. WHO and partners are supporting 14 emergency medical teams (36 deployed personnel) across 21 hospitals, supplying trauma and surgical kits, ICU ventilators, and technical guidance including an NCD medication substitution guide and trainings to sustain trauma, NCD, maternal and critical care services [1067][1069].

Critical gaps persist in essential medicines and supplies (trauma kits, EmONC items), blood bank capacity, diagnostics, trained surgical and emergency personnel, and weakened referral pathways. Eleven percent of partners are at high risk and 53% at moderate risk of service interruption within 4–8 weeks absent new funding [1066][1067][1070]. Coordination is led by the Ministry of Public Health with Health Sector and Public Health Emergency Operations Centre support, with active coordination between government, UN agencies and NGOs to sustain mobile outreach and facility reactivation [1072][1067][1073].

Nutrition

Availability and Access to Nutrition Services

Action Against Hunger (ACF) assessments (March–May 2026) document severe disruption of infant and young child feeding (IYCF), with exclusive breastfeeding rates critically low — 7% in Saida, 16% in BaalbekHermel, and 24% in Beirut and Mount Lebanon — and reliance on breastmilk substitutes (BMS) reaching 85% in Saida, 59% in Beirut and Mount Lebanon, and 38% in Bekaa/BaalbekHermel [1116][1117].

These conditions are driven by overcrowded shelters, lack of privacy and breastfeeding support (including absence of pumps and private spaces), and inadequate WASH infrastructure for safely preparing formula [1116]. ACF estimated approximately 12,200 displaced pregnant women nationwide, including around 675 in collective shelters, and identified at least 200 children aged 0–3 in Rmeish and Ain Ebel needing IYCF support, while finding that 38% of collective shelters have no nutrition coverage, with the largest gaps in Beirut and Mount Lebanon [1116][1117].

Key service delivery figures reported by mid-May include: since the onset of escalations, 19,869 children under five, adolescent girls, and pregnant and lactating women received emergency nutrition rations and micronutrient supplementation (Nutrition Sector) [1118][1120]; the national IYCF hotline received 522 calls and referred 394 cases for specialized support, and more than 11,799 caregivers benefited from IYCF/nutrition counselling and awareness activities by 14 May [1120][1118]; the Nutrition Sector/MoPH distributed 1,277 tins of unbranded infant formula to 154 of 610 assessed infants and provided 84 BMS kits to support safer formula preparation in shelters [1120][1118]; ACF's nutrition team provided supplements to 925 children (6–59 months) and 111 pregnant and breastfeeding women, and delivered IYCF counselling to 157 pregnant women/caregivers [1116]; International Medical Corps nutrition teams screened 739 children under five and pregnant/breastfeeding women, with 388 people receiving nutritional supplementation, while 35 mobile medical units reached 125 shelters and community sites [1134][1132]; and primary healthcare outreach continues through 211 outreach units linked to 185 PHCCs [1120].

Health system constraints — including multiple hospitals and dozens of PHCCs closed or operating at reduced capacity — are overburdening remaining services and limiting maternal and child nutrition outreach and referrals [1136]. Access constraints in parts of southern Lebanon and Nabatieh, UXO contamination, damaged infrastructure, and funding shortfalls for the Lebanon Flash Appeal (approximately 42% funded as of mid-May) further limit coverage and scale-up of nutrition services [1131][1124].

Nutrition Needs and Current Response

Acute food insecurity is rising sharply: an IPC projection estimates approximately 1.24 million people in Lebanon will face acute food insecurity between April and August 2026, with Crisis (IPC Phase 3) outcomes expected to persist across multiple governorates amid disrupted markets and livelihoods [1121][1138]. Nutrition needs are especially severe among infants, young children, and pregnant and lactating women, with IYCF practices deeply compromised as documented by ACF assessments conducted between March and May 2026 [1116][1117].

The Nutrition Sector and partners have mounted a multi-pronged response since the onset of escalations. Emergency nutrition rations and micronutrient supplementation reached 19,869 children under five, adolescent girls, and pregnant and lactating women [1118][1120]. Targeted infant feeding support scaled up notably between 11 and 14 May: unbranded infant formula distributed rose from 948 tins provided to 113 of 567 assessed infants to 1,277 tins provided to 154 of 610 assessed infants, and BMS safety kits increased from 70 to 84 over the same period [1118][1120]. IYCF hotline activity also expanded during this period, with calls rising from 481 to 522 and referrals from 357 to 394, while caregivers reached through counselling and awareness activities grew from 10,508 to 11,799 [1118][1120].

Key gaps continue to undermine the response. Coverage outside collective shelters remains limited, and 38% of collective shelters still lack nutrition services [1116][1118][1120]. Closures and reduced functionality of health facilities constrain maternal and child nutrition services [1121][1131]. Most critically, major funding shortfalls persist: approximately US\$129 million of the US\$308 million Flash Appeal had been received as of mid-May, threatening the continuity and scale-up of nutrition interventions [1118][1120][1121][1131].

WASH

Availability of Water Sources

WASH partners have sustained public water supply by providing 803,000 litres of fuel to Water Establishments—supporting an estimated 656,973 people—and by supporting 256 water stations with fuel, repairs and spare parts to maintain network functionality under increased demand [1161]. Partners also delivered 3,765,725 litres of potable water and 76,090 m³ through water trucking to sites with limited or disrupted supply [1161]. WASH coverage across open collective shelters was reported at 96.7%, and since 2 March partners have delivered WASH assistance in 611–615 collective shelters [1166][1161].

Environmental assessments documented damage to irrigation networks and the destruction of approximately 20 local water collection basins, and warned that debris and pollutants from strikes risk contaminating wells and groundwater, complicating safe water access in affected areas [1192]. Overcrowded shelters with water leakage, blocked drainage and sewage contamination have been identified as further threats to safe water access, driving repeated infections and raising malnutrition risks among infants and young children [1173].

Funding shortfalls—with the Flash Appeal approximately 42% funded by mid-May—and access constraints including insecurity, damaged roads, UXO and infrastructure damage were flagged as major risks to sustaining public water systems and water provision in collective shelters and hard-to-reach areas [1165][1161][1160].

Access to Sanitation Facilities and Hygiene Practices

Since 2 March, WASH partners expanded sanitation and hygiene assistance across collective shelters, reaching 611 sites as of 11 May and 615 sites as of 14 May [1166][1161]. Key outputs delivered include:

- 95,954 hygiene kits, 52,803 menstrual hygiene kits and 1,394 cleaning kits distributed to collective shelters [1161].
- Installation of 92 external latrines and 333 external showers in and around shelters, plus emergency desludging services [1161].

Mobile medical and NGO teams supplemented these distributions at shelter level, with International Medical Corps distributing 976 hygiene kits to address urgent needs in displacement sites [1178].

Shelter rehabilitation efforts also included upgrades to toilets, plumbing and accessibility improvements for persons with disabilities [1164].

Field assessments by Action Against Hunger documented water leakage, blocked drainage, sewage contamination, overcrowded and unhygienic shared sanitation facilities, and repeated infections among infants and young children in collective shelters [1173]. Save the Children reported thousands of children in collective sites exposed to inadequate WASH conditions and rising hygiene-related problems such as scabies [1210]. WHO conducted ongoing water safety monitoring, installation and repair of water filters, hygiene education and field visits to shelters and primary health care facilities to reduce outbreak risks and sustain services [1196].

Despite scaled deliveries, partners warned of sustainability risks as funding shortfalls and pipeline pressures in mid-May threaten continuity of shelter WASH services [1165][1160].

Waste Management and Vector-Borne Disease Risks

Assessments and media reporting note large volumes of rubble containing asbestos, paints, solvents, heavy metals and other hazardous materials, with dust generation and contaminant migration posing risks to soil, wells and people involved in clean-up and return processes [1192][1197]. Municipal waste-management and debris-removal capacity is reported as strained, increasing the likelihood of uncontrolled disposal and contamination; assessments call for hazardous-waste sorting and safe asbestos removal during clearance and reconstruction operations [1212][1192][1197].

No major disease outbreaks have been reported in shelters to date, though health partners warn that overcrowding, limited water access and inadequate sanitation sustain significant risk for waterborne and vector-borne disease transmission [1167][1166].

Main WASH Needs and Humanitarian Response

Humanitarian actors identify urgent needs to restore public water supply, rehabilitate and expand sanitation infrastructure in collective shelters and return areas, and scale up hygiene promotion and water quality interventions to reduce disease risk in overcrowded sites [1166][1161][1175]. Immediate operational priorities cited by partners and authorities include installation of water filtration systems, urgent fuel support for water systems, and rehabilitation and expansion of sanitation facilities, with WHO and others supporting water safety monitoring, filter repairs and installation, and hygiene promotion in shelters [1166][1196].

These combined interventions—water trucking, fuel provision, station repairs, latrine and shower installation, emergency desludging, kit distributions and hygiene education—have maintained high WASH coverage in collective shelters (96.7%) and reduced reliance on costlier emergency modalities by sustaining network functionality for hundreds of thousands of people [1166][1161][1196].

Scaling the response faces major constraints: critical gaps in sanitation infrastructure identified by baseline assessments, continuous population movements increasing pressure on services, access and logistics impeded by insecurity, damaged roads and UXO (especially in southern Lebanon and Nabatieh), and funding shortfalls with the Flash Appeal only approximately 42% funded by mid-May, putting WASH services at risk of interruption [1166][1165][1160][1162].

Shelter and Non-Food Items (NFI)

Damage to Housing and Shelter Needs

Since the truce, 10,632 housing units have been registered as affected—about 5,386 completely destroyed and 5,246 partially damaged—and earlier CNRSL data identified 37,836 housing units damaged or destroyed between 2 March and 7 April 2026 [1232]. Many damaged dwellings have lost roofs, windows and basic services (water, electricity) or have structural cracks, rendering otherwise standing houses uninhabitable [1232]. More than one million people remain displaced overall, with roughly 129,700 people sheltering in some 632 collective sites as of mid-May, while the vast majority of displaced households are hosted outside formal shelters in rented accommodation, with host families or informal arrangements [1222][1224]. Collective sites host high numbers of vulnerable people, including approximately 44,800 children, 6,700 older persons and 2,699 people with disabilities, with further disaggregation indicating about 1,200 people with physical disabilities, 689 with mental disabilities, 422 with hearing impairments and 388 with visual impairments [1230]. Health partners report inadequate meal frequency among displaced pregnant women in collective shelters, and assessments estimate some 12,200 pregnant women are currently displaced nationwide [1230][1234]. Reconstruction and safe return are constrained by access restrictions (including the so-called "Yellow Line"), unexploded ordnance contamination, damaged roads and bridges, loss of ownership documentation complicating claims, and limited financing and materials for repairs [1232][1226][1222]. Schools remain used as collective shelters and multiple health facilities remain nonoperational—including three hospitals and 42 primary healthcare centres reported closed—reducing options for durable shelter and essential services in return areas [1252][1222]. Repeated, fluid movements and secondary displacements are increasing overcrowding, protection risks and the complexity of targeting shelter interventions and tracking needs across governorates [1230][1226].

Living Conditions in Camps or Settlements

Living conditions in collective sites remain overcrowded and substandard, with reports of damp rooms, poor ventilation, sewage contamination, water shortages and limited privacy, increasing exposure to infectious disease and malnutrition risks—particularly among children, pregnant and lactating women, and persons with disabilities [1233][1222][1230]. WASH support has been scaled up since 2 March, with partners providing assistance in some 611–615 collective shelters, distributing 88,043 hygiene kits, 50,333 menstrual hygiene kits and 1,387 cleaning kits, delivering approximately 3,765,725 litres of potable water, and installing 85 external latrines and 314 external showers, with shelter WASH coverage reported at roughly 96.7% where services are maintained [1226][1244][1222].

Health services are severely constrained: attacks on healthcare have continued since 2 March, several hospitals and numerous primary health centres remain closed or operating at reduced capacity, and partners warn of service interruptions and heightened outbreak risk in overcrowded shelters [1223][1241][1244]. Protection risks are elevated—women and girls in tents or public spaces face urgent GBV and safety needs, family separation and disrupted continuity of care are increasing child protection vulnerabilities among hundreds of thousands of displaced children, and UXO contamination, insecurity and infrastructure damage are impeding safe returns and site safety assessments [1222][1230][1226].

NFI and Shelter Needs and Current Response

Shelter partners have supported 44,106 households through distribution of 127,028 mattresses, 166,816 blankets and 67,054 pillows, alongside sleeping mats, solar lamps and jerrycans, with distributions occurring both within and outside collective shelters [1222]. Partners are undertaking repairs and upgrades across 385 collective shelters, of which 286 have been completed to improve safety, capacity and privacy for displaced populations [1222]. UNHCR is repairing and rehabilitating 65 shelters to benefit about 13,006 displaced people, having completed works in 33 sites to date and installed partitioning and accessibility measures to enhance privacy and inclusion [1224][1226]. Initial core relief item (CRI) distributions reached all preidentified households in collective shelters, while IOM has distributed CRIs to 32,340 individuals across locations and referred 3,315 people in nongovernment managed sites to shelter, NFI, food, cash and medical assistance through site management coordination mechanisms [1238][1222]. On 11 May, more than 4,500 emergency shelter and household items were airlifted to Beirut via the EU Humanitarian Air Bridge for distribution by IOM and partners [1240][1223]. Funding and logistics remain key constraints: as of midMay the 2026 Lebanon Flash Appeal had received about US\$129 million of US\$308 million required (approximately 42% funded), limiting partners' ability to replace depleted CRI stocks and scale shelter and NFI response, while repeated, fluid displacement is leading to items being left behind and increasing demand for replenishment [1223][1226]. Access constraints, infrastructure damage and supplychain disruptions in parts of southern Lebanon and Nabatieh are impeding humanitarian movements and risking pipeline breaks and service interruptions that would affect continued NFI and shelter support [1223][1238][1237]. The Shelter Sector has engaged the Ministry of Social Affairs and the Ministry of Education to plan transitions from emergency collective shelters toward more sustainable solutions, while MoSA, with IOM and the SMC subsector, is scaling up site management and mapping partner capacities to align site care and maintenance activities [1222][1226].

Protection

Impact on civilians

No data available for this section.

Factors Driving protection risks

Ongoing airstrikes and hostilities across southern Lebanon, the Bekaa Valley and Beirut's southern suburbs continue to be the primary driver of civilian casualties, displacement and protection risks [1283]. Legal vulnerability and lack of civil documentation are limiting mobility, access to services and legal protection for displaced populations and returnees, with missing or damaged documentation reported as a frequent barrier at arrival and return locations [1311][1273][1268].

Severe economic deprivation, disrupted livelihoods and depleted savings are forcing negative coping mechanisms—including rising child labour, increased street-based work, and early marriage—while also increasing GBV and child protection concerns and exacerbating malnutrition risks among infants, young children and pregnant and lactating women [1268][1269][1293]. The breakdown of social structures and overcrowded, inadequate accommodation conditions further compound risks for the most vulnerable groups, including persons with disabilities, older persons, women and girls, separated children, and people outside formal shelters [1279][1268].

Limited humanitarian reach, operational constraints in parts of the South and Nabatieh, and critical funding shortfalls—with the Lebanon Flash Appeal approximately 38–42% funded in midMay—are reducing protection coverage outside collective shelters and hindering scaleup of durable solutions and specialized services [1273][1274][1291].

Psychological and mental health impacts

According to UNICEF, an estimated 770,000 children in Lebanon are experiencing heightened psychological distress from repeated exposure to violence, loss and displacement. UNICEF's 2025 Child-focused Rapid Assessment found 72% of caregivers reported their children were anxious or nervous and 62% reported children were depressed or sad [1276][1287]. Displacement, overcrowding in collective shelters and disrupted basic services are further aggravating psychosocial needs across affected communities [1278][1279]. Children repeatedly displaced or exposed to attacks, as well as displaced persons with pre-existing vulnerabilities including people with mental disabilities, face elevated risk of chronic mental health conditions [1276][1287][1279][1310].

The International Rescue Committee reported widespread household reports of overwhelming anxiety, grief and depression and warned of declining funding for lifesaving mental health services, underscoring a critical gap between needs and available resources [1310][1306]. Health and humanitarian actors report ongoing MHPSS delivery but major gaps and constrained coverage due to facility closures, attacks on health care and limited funding [1289][1268][1306]. The National Mental Health Programme and WHO are running specialized self-care and burnout-prevention trainings for frontline workers—eight sessions across Beirut, Mount Lebanon, the Bekaa, the South and the North in May—to address provider stress and capacity [1289][1306].

Key MHPSS reach figures include:

- 68,528 people reached through MHPSS interventions as of 14 May [1271][1268].
- UNHCR/partners provided MHPSS to 4,971 individuals through community and shelter-based modalities [1269].

- IOM reached 4,159 people with MHPSS services since 2 March [1304].
- International Medical Corps teams reached 660 people with psychological first aid and non-specialized support [1301].
- MoPH/WHO reporting shows 4,138 IDPs received mental health medications and the national lifeline (1564) recorded 3,825 cumulative calls as of 14 May [1279].

Protection Needs and Current Response

Protection risks are acute for women and girls, separated children, persons with disabilities, and people outside collective shelters, who face insecure accommodation, severe WASH gaps, legal and documentation vulnerabilities, and heightened exposure to exploitation and harmful coping mechanisms [1271][1268][1279]. Coverage outside collective sites remains limited—only about 39% of people outside sites have been reached to date—and shortages in protection staffing, disrupted case follow-up and legal/documentation gaps are constraining effective protection outcomes and referrals [1269][1268]. The Lebanon Flash Appeal was roughly 38% funded in early May and about 42% funded by mid-May, a funding shortfall that limits partners' ability to scale and sustain protection and referral services [1269][1291].

OCHA reported that 77 Protection/Child Protection/GBV partners reached 192,600 IDPs by 14 May (61% inside collective sites) [1271][1268]. Key service delivery figures as of 14 May include:

- 1,320 at-risk children and adults received dedicated case management [1271][1268].
- 1,925 protection cash grants provided [1271][1268].
- 25,353 women and girls received dignity kits [1271][1268].
- Approximately 90,000 people reached via one-way protection messages and 21,000 supported through protection hotlines and feedback mechanisms [1271].
- UNHCR reported reaching over 24,300 people with direct protection support and issuing more than 2,000 Emergency Protection Cash Assistance grants, reaching over 9,700 individuals [1269][1273].
- IOM reported providing protection assistance to 6,372 individuals and referring 3,315 people in non-government managed sites to essential services [1304].

Community outreach and awareness activities reached 89,651 people on topics including bullying, GBV and safety risks [1268]. Humanitarian actors are prioritizing expanded dissemination of referral pathways, scaling community-based protection including Community Development Centres, a nationwide GBV assessment with publication expected by end-May, and mainstreaming safeguarding and PSEAH across health and protection operations with WHO support to strengthen survivor-centred approaches and reporting mechanisms [1269][1278][1268]. Sector updates indicate specialized child protection and referral pathways remain insufficient and call for expanded staffing and wider dissemination of referral information to improve access to services [1271][1268].

Child Protection concerns

Approximately 390,000 children remain displaced, with family separation, loss of housing and income heightening risks of violence, exploitation, neglect and harmful coping mechanisms including child labour [1268]. UNICEF reported that at least 59 children were killed or injured in the week to 13 May, and that 200 children have been killed and 806 injured since 2 March—including 23 killed and 93 injured since the 17 April ceasefire [1276][1287][1327]. UNHCR and sector reporting note rising child labour, increased street-based work, and early marriage as negative coping mechanisms among displaced families [1269][1268]. Protection and child protection capacity is strained by shortages of CP and GBV staff, limiting reach and complicating case follow-up amid repeated displacement [1268][1271].

Action Against Hunger's March–May 2026 assessments found severely disrupted infant and young child feeding practices—exclusive breastfeeding reported as low as 7% in Saida—and high reliance on breast-milk substitutes. ACF reported supporting 925 children aged 6–59 months with supplements and providing infant and young child feeding (IYCF) counselling to 157 caregivers during that period [1292][1293].

Violence against women and girls

Protection actors report acute GBV risks for women and girls amid widespread displacement, overcrowded collective sites and limited safe accommodation. GBV risks include increased domestic violence linked to economic stress, harassment limiting women's mobility, and rising early and forced marriage as a negative coping mechanism [1271][1268][1269]. Repeated displacement is disrupting case follow-up and continuity of care, while funding and operational constraints are limiting protection reach outside collective shelters [1291][1268][1269].

UNHCR led a nationwide GBV assessment comprising 37 focus group discussions and 21 key informant interviews, with data collection closed on 8 May and publication expected by end-May to inform prioritization of prevention and response actions [1269]. Of 21 Community Development Centres, 19 remained operational and reached 9,724 displaced people—63% of whom were women and girls—through outreach, MHPSS and protection information [1269]. WHO deployed missions and guidance to strengthen PSEAH and GBV mainstreaming and safeguarding within the health emergency response to mitigate risks of sexual exploitation, abuse and harassment [1279][1278].

Despite substantial GBV and MHPSS inputs delivered by partners, critical gaps persist in coverage outside shelters, staffing constraints, disrupted case continuity due to repeated displacement, and funding shortfalls that hinder further scale-up [1271][1268][1269][1291].

Vulnerable Groups / Special Needs Groups

Unaccompanied and Separated Children

Approximately 390,000 children remain displaced, and family separation, loss of housing and livelihoods have increased risks of children being exposed to violence, exploitation, neglect and harmful coping mechanisms including child labour [1344]. UNICEF reported that at least 59 children were killed or injured in the seven days to 13 May and that since 2 March some 200 children have been killed and 806 injured, while an estimated 770,000 children are experiencing heightened psychological distress [1353]. Overcrowded shelters and people living in public or open spaces face inadequate WASH, disrupted schooling, outbreaks of skin disease (including scabies) and rising psychosocial distress among children [1345][1356][1353].

Protection, child protection and GBV actors (77 partners) scaled up child-focused outreach, safe spaces and psychosocial support, reaching 192,600 IDPs nationwide as of 14 May and delivering community outreach, awareness on bullying/GBV/safety and IYCF counselling and referrals [1344]. As of 11 May, partners reported reaching 182,313 IDPs, delivering MHPSS to 64,976 people, providing 1,259 dedicated case management services, issuing 1,796 protection cash grants and distributing 24,841 dignity kits to women and girls [1347]. UNHCR and nine protection partners reached more than 24,300 people with targeted protection interventions including child protection, safe spaces and structured activities in collective shelters [1349].

Infant and young child feeding support included a national IYCF hotline receiving 522 calls (394 referrals), 11,799 caregivers reached with IYCF/nutrition/ECD counselling, targeted distribution of formula to identified infants, and Action Against Hunger reported supporting 925 children (6–59 months) and 111 pregnant/breastfeeding women with supplements while 157 pregnant women/caregivers received IYCF counselling; ACF identified at least 200 children aged 0–3 in isolated areas (Rmeish and Ain Ebel) needing nutrition support [1344][1351].

Repeated displacement, fragmented referral pathways and funding and access constraints are disrupting continuity of care and complicating case follow-up, limiting partners' ability to scale up protection, tracing and specialized services for separated children [1344][1347][1352].

Pregnant and Lactating Women

Action Against Hunger's March–May 2026 field assessment across Beirut, Mount Lebanon, Bekaa, Baalbek-Hermel and the South estimated approximately 12,200 pregnant women displaced nationwide, including at least 675 living in collective shelters, and carried out Infant and Young Child Feeding (IYCF) assessments with 433 caregivers [1351][1350]. ACF reported many pregnant and lactating women consuming only one to two meals per day with limited dietary diversity and recorded cases of low MUAC among lactating women, indicating heightened risk of maternal undernutrition and adverse birth outcomes [1351][1350]. Uncontrolled donations and distributions of breast-milk substitutes were observed in at least one collective shelter (Saida), posing risks to safe infant feeding in displacement settings [1351].

Health service availability is severely constrained: multiple reports indicate three hospitals and some 41–42 primary health care centres remain closed, six PHCCs are operating with restricted services, and six hospitals in Tyre, Bint Jbeil/Bent Jbeil, Hasbaya and Nabatieh have not resumed delivery services – limiting maternal health access in these districts for almost two months [1354][1344][1369]. WHO and partners reported that since 28 February health actors have delivered 269,748 consultations, including 4,359 antenatal and 870 postnatal consultations, and provided 585 hospitalization supports (116 institutional deliveries) as part of the emergency response to date [1370]. Mobile medical teams have maintained some access, with 35 mobile units reaching 125 shelters; International Medical Corps teams screened 739 children under five and pregnant or breastfeeding women, with 388 people receiving nutritional supplementation through those activities [1375][1380].

Nutrition partners report scaled emergency nutrition support: since the onset of escalations, a total of 19,869 children under five, adolescent girls and pregnant and lactating women received one month of emergency nutrition rations and micronutrient supplementation. The Nutrition Sector distributed targeted unbranded infant formula tins and BMS kits to identified infants in need – 1,277 tins to 154 assessed children and 84 BMS kits as of 14 May [1347][1344].

Humanitarian access and funding shortfalls are constraining continuity of maternal services: health partners warned services could be interrupted without additional funding, the Health Sector called for an urgent US\$37 million top-up to sustain operations, and the Lebanon Flash Appeal was approximately 42% funded (US\$129 million of US\$308 million) as of mid-May 2026 [1370][1366][1352].

Adolescents and Youth

Children and adolescents face major disruption to schooling, high exposure to violence and displacement, rising psychosocial needs, and limited pathways to sustainable employment in Lebanon [1344][1347]. As of 14 May, approximately 164,059 displaced children accessed formal education online, 94,067 attended in-person classes, and 4,222 were supported through non-formal education, while 1,694 education kits reached some 84,700 children; 764 children received individual student kits and 49 children with disabilities received assistive devices [1344]. The Ministry of Education and Higher Education confirmed national examinations will proceed with registration open until 15 May and allowed Syrian and other non-national students to register on submission of certified documents [1344]. Online learning continuity remains constrained by poor connectivity and limited access to devices, prompting MEHE to prioritise in-person learning and to approve partner-run activities inside shelters [1344].

Protection and child-protection partners have scaled up psychosocial responses: as of 11 May, 77 protection/child-protection/GBV partners reached 182,313 IDPs, while partners report rising risks including family separation, child labour and other exploitative coping mechanisms [1347][1344]. UNHCR provided MHPSS to 4,971 individuals (4–10 May), IOM reported reaching 4,159 people with MHPSS services (reported 7–13 May), and IMC mental-health teams reached about 660 people by 13 May [1349][1374][1380].

WHO and the National Mental Health Programme are rolling out self-care and burnout-prevention trainings for frontline workers across Lebanon throughout May 2026, and WHO has deployed a regional MHPSS consultant to strengthen multisectoral coordination and service delivery [1347][1364][1366].

Vocational and cash-plus initiatives exist but face major limits: Oxfam's Tripoli cash-plus intervention targeted 475 households, with roughly 150 women selected for vocational training and about 50 progressing to work-based learning, yet training gains frequently did not translate into sustained employment due to weak labour demand, mobility and caregiving constraints; programme supports included a transport allowance of about US\$2 per day and a US\$150 monthly incentive during work-based learning [1405]. These education, livelihood and mobility constraints are increasing reliance on negative coping mechanisms among adolescents [1405][1344].

People with Disabilities

In mid-May 2026, humanitarian reporting noted a noticeable increase of persons with disabilities in collective shelters and highlighted elevated protection risks and disrupted continuity of care for this group [1369][1344]. Shelter monitoring dashboards list specific disability caseloads in collective sites: 1,200 people with physical disabilities, 689 with mental disabilities, 422 with hearing impairments and 388 with visual impairments, with an aggregate figure of 2,699 people with disabilities in shelters [1369]. Disability-specific services have been cut: five rehabilitation centres ceased services, ten specialised schools for children with disabilities closed, and an estimated 113,000 persons with disabilities lost subsidised access to healthcare [1377].

Health partners reported severe mid-May constraints to service continuity – repeated attacks on healthcare, multiple hospital and PHC closures, and restricted maternity and specialised services – undermining access to rehabilitation, chronic care and assistive-device maintenance for persons with disabilities [1367][1354][1366]. The Shelter Sector has identified 17 collective shelters where accessibility adaptations were implemented and is planning to scale up adaptations based on referrals and site profiles [1349]. UNHCR reported reaching over 24,300 people with protection services, with emergency protection cash assistance reaching more than 9,700 people, including over 700 persons with disabilities, while community centres and border mobile teams provide referrals and immediate assistance for people with specific needs [1362].

UXO contamination, damaged infrastructure and access constraints continue to impede safe returns and heighten mobility and protection hazards for persons with disabilities [1347][1361][1369]. Major funding shortfalls for the Flash Appeal (roughly 38–42% funded in mid-May) threaten the ability to sustain and scale disability-inclusive adaptations and services [1352][1360][1347].

Elderly

A noticeable increase in older persons in collective shelters has been observed, with DRM dashboards reporting 6,700 older people among approximately 129,724 IDPs in shelters as of 14 May 2026 [1369]. Humanitarian partners report rising socioeconomic and protection pressures that increase older persons' vulnerability to mobility constraints, exclusion from services, and reliance on negative coping mechanisms amid funding and access constraints [1347][1369].

Health service availability is severely constrained for older people who disproportionately rely on chronic disease and routine care: three hospitals and 41–42 primary health care centres remain closed and an additional six PHCCs are operating at restricted capacity [1366][1344]. Primary healthcare outreach and mobile units are active but limited in coverage – 211 outreach units linked to 185 PHCCs continue operations and mobile teams have reached collective sites (35 mobile units reached 125 shelters), providing consultations, medications and chronic disease support [1344][1375]. Coverage gaps persist despite these efforts: 110,069 IDPs received medications and 48,851 IDPs received chronic medication support, indicating ongoing but insufficient reach for people with NCDs who are disproportionately older [1344].

Shelter accessibility adaptations remain limited but are being scaled up – 17 collective shelters have implemented accessibility adaptations – while protection monitoring continues to highlight structural barriers and heightened risks for older persons in shelters [1349][1382][1369].

Displaced

More than one million people are displaced in Lebanon, with 1,049,328 registered through the government relief platform (MoSA) as of 1 April 2026 [1360]. Approximately 129,721 people were reported sheltering in 632 collective shelters as of 14 May 2026, with shelter occupancy and demand rising following secondary displacement and repeated movements [1352]. Most displaced households remain outside formal shelters in host families, rented or informal arrangements [1371][1369][1352]. Displaced households face severe financial strain, rising rents and eviction risks outside shelters, constrained livelihood opportunities, and increasing reliance on negative coping (borrowing, debt, early marriage), limiting feasible local integration or durable solutions [1349].

Collective shelters are overcrowded and substandard in many locations; shelter partners are repairing and upgrading sites (360 sites targeted, 282 completed to date) while new shelters have been opened to absorb arrivals [1347]. Returns remain limited and frequently non-sustained: many families attempting to return have been forced back to collective sites because of widespread damage, insecurity and infrastructure constraints, complicating safe, voluntary and dignified return prospects [1344][1352].

Funding shortfalls are constraining the response: the 2026 Lebanon Flash Appeal was around 42% funded (approximately US\$129 million of US\$308 million) in mid-May, placing critical services including WASH at risk of interruption [1352]. Migrants and third-country nationals are among the most vulnerable displaced populations, facing language, information and status barriers to assistance; IOM has assisted dozens of migrants with voluntary return and received hundreds of evacuation requests during the crisis [1374][1373].

Minorities

Protection partners report heightened risks for marginalized groups — including women, children, persons with disabilities and those lacking documentation — who face housing insecurity, substandard displacement conditions and disrupted continuity of care [1344][1347]. Migrants face language barriers and concerns linked to migration status that limit access to shelter and assistance; IOM reported over 700 evacuation requests since March, with 41 migrants returned with IOM support and more than 150 cases being processed [1373].

UNHCR and partners are providing legal assistance and civil-documentation support to returnees and refugees, with UNHCR reporting it had reached over 24,300 people with protection support and issued more than 2,000 Emergency Protection Cash Assistance grants reaching about 9,700 people, including over 700 persons with disabilities, by early May [1362]. A nationwide GBV assessment covering all governorates included displaced and host community women and girls, persons with disabilities and LGBTIQ individuals; data collection closed on 8 May and publication was expected by end-May, reflecting efforts to identify and prioritise marginalised groups in protection planning [1349].

Lebanon's fragmented social protection system and clientelist access patterns — shaped by political affiliation and local networks — constrain equitable access for minorities and limit longer-term integration, even as initiatives such as the Tripoli Peace Initiative and cash-plus programmes aim to reach marginalised communities [1376][1387][1405]. Humanitarian leadership has called for more inclusive, decentralised responses and stronger engagement with municipalities to support safe, dignified returns and better include affected communities in recovery planning [1352]. Persistent access constraints, infrastructure damage and significant funding shortfalls in mid-May are limiting partners' ability to sustain services and reach marginalised groups in hard-to-reach and return areas [1352][1344].

Operational Environment

Economy

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